

Patient Name : Mrs.ASMITA DASGUPTA
Age/Gender : 51 Y 7 M 15 D/F
UHID/MR No : FSBP.0000018456
Visit ID : FSBROPV29438
Ref Doctor : Self
Emp/Auth/TPA ID : 123

Collected : 01/Sep/2025 09:34AM
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Status : Final Report
Sponsor Name : SGD 369 HCL

DEPARTMENT OF HAEMATOLOGY

AHC ABOVE 40Y FEMAL - SARAT BOSE ROAD

Test Name	Result	Unit	Bio. Ref. Interval	Method
BLOOD GROUP ABO AND RH FACTOR , WHOLE BLOOD EDTA				
BLOOD GROUP TYPE	B			Forward Grouping with Slide
Rh TYPE	POSITIVE			Forward Grouping with Slide

Comment:

1. This tests determines ABO & Rh blood groups (testing for other blood group systems not performed) through immunological reaction between RBC antigen & antibody.
2. ABO system also has Subgroups of A, B and rare phenotype as Bombay blood group which requires further testing and required recommendations as per the case will be provided.
3. Rh system in certain individual can have weak or partial Rh D expression which can result in weaker agglutination reactions and hence all Rh D Negative groups need to be further cross verified using Rh Du testing.
4. In case of Newborn - Only forward typing is performed, reverse typing is not performed, since the antibodies are not fully formed. Hence it is recommended to re-test blood grouping after 6 months.
5. In certain cases History of Recent blood transfusion (within 3-4mths), of bone marrow transplantation, certain drugs (especially monoclonal antibody) & certain malignancies may interfere with interpretation of blood grouping.
6. It is always recommended for reconfirmation of the Blood Group along with cross matching before blood transfusion.

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Test Name	Result	Unit	Bio. Ref. Interval	Method
HEMOGRAM.- , WHOLE BLOOD EDTA				
HAEMOGLOBIN	12.9	g/dL	11-15	Colorimetric
RBC COUNT	4.37	10 ¹² /L	3.5-5.5	IMPEDANCE
PCV	39.5	%	36-48	ANALYSER CALCULATION
MCV	92.5	fL	80-99	ANALYSER CALCULATION
MCH	29.5	pg	26-32	ANALYSER CALCULATION
MCHC	32.6	g/dL	30-36	ANALYSER CALCULATION
R.D.W- CV	12.3	%	11.5-14.5	ANALYSER CALCULATION
R.D.W-SD	36.4	fL	37-54	ANALYSER CALCULATION
TOTAL LEUCOCYTE COUNT (TLC)	4	10 ⁹ /L	4-10	IMPEDANCE
DIFFERENTIAL LEUCOCYTIC COUNT (DLC)				
NEUTROPHILS	62	%	40-80	Microscopy
LYMPHOCYTES	35	%	20-40	Microscopy
MONOCYTES	02	%	2-10	Microscopy
EOSINOPHILS	01	%	1-6	Microscopy
BASOPHILS	00	%	0-1	Microscopy
Neutrophil lymphocyte ratio (NLR)	1.77			Calculated
PLATELET COUNT	150.00	10 ⁹ /L	100-300	Electrical impedance/MICROSCOPY
MPV	10.6	fL	7.4-10.4	Calculated
PDW	14.5	—	10.0 - 17.0	ANALYSER CALCULATION

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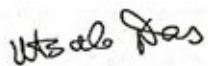
DEPARTMENT OF HAEMATOLOGY

AHC ABOVE 40Y FEMAL - SARAT BOSE ROAD

ERYTHROCYTE SEDIMENTATION RATE (ESR)	16	mm/h(1st Hr.)	0-20	Method (Westergren & Manual)
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PERIPHERAL SMEAR

RBC NORMOCYTIC NORMOCHROMIC
WBC WITHIN NORMAL LIMITS
PLATELETS ARE ADEQUATE ON SMEAR



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DEPARTMENT OF BIOCHEMISTRY

AHC ABOVE 40Y FEMAL - SARAT BOSE ROAD

Test Name	Result	Unit	Bio. Ref. Interval	Method
GLUCOSE, FASTING , NAF PLASMA	84	mg/dL	70-110	GOD- Trinder

Comment:

As per American Diabetes Guidelines, 2023

Fasting Glucose Values in mg/dL	Interpretation
70-100 mg/dL	Normal
100-125 mg/dL	Prediabetes
≥126 mg/dL	Diabetes
<70 mg/dL	Hypoglycemia

Note:

- 1.The diagnosis of Diabetes requires a fasting plasma glucose of $>$ or $=$ 126 mg/dL and/or a random / 2 hr post glucose value of $>$ or $=$ 200 mg/dL on at least 2 occasions.
2. Very high glucose levels ($>$ 450 mg/dL in adults) may result in Diabetic Ketoacidosis & is considered critical.

Test Name	Result	Unit	Bio. Ref. Interval	Method
GLUCOSE, POST PRANDIAL (PP), 2 HOURS , SODIUM FLUORIDE PLASMA (2 HR)	129	mg/dL	90-140	GOD- Trinder

Comment:

It is recommended that FBS and PPBS should be interpreted with respect to their Biological reference ranges and not with each other.

Conditions which may lead to lower postprandial glucose levels as compared to fasting glucose levels may be due to reactive hypoglycemia, dietary meal content, duration or timing of sampling after food digestion and absorption, medications such as insulin preparations, sulfonylureas, amylin analogues, or conditions such as overproduction of insulin.

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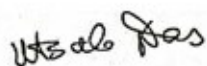
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AHC ABOVE 40Y FEMAL - SARAT BOSE ROAD

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DEPARTMENT OF BIOCHEMISTRY

AHC ABOVE 40Y FEMAL - SARAT BOSE ROAD

Test Name	Result	Unit	Bio. Ref. Interval	Method
HBA1C , WHOLE BLOOD EDTA				
HBA1C	5.2	%	4.5-6.5	HPLC
ESTIMATED AVERAGE GLUCOSE (eAG)	103	mg/dL		Calculated

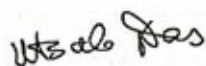
Comment:

Reference Range as per American Diabetes Association (ADA) 2023 Guidelines:

REFERENCE GROUP	HBA1C %
NON DIABETIC	<5.7
PREDIABETES	5.7 – 6.4
DIABETES	≥ 6.5
DIABETICS	
EXCELLENT CONTROL	6 – 7
FAIR TO GOOD CONTROL	7 – 8
UNSATISFACTORY CONTROL	8 – 10
POOR CONTROL	>10

Note: Dietary preparation or fasting is not required.

- HbA1C is recommended by American Diabetes Association for Diagnosing Diabetes and monitoring Glycemic Control by American Diabetes Association guidelines 2023.
- Trends in HbA1C values is a better indicator of Glycemic control than a single test.
- Low HbA1C in Non-Diabetic patients are associated with Anemia (Iron Deficiency/Hemolytic), Liver Disorders, Chronic Kidney Disease. Clinical Correlation is advised in interpretation of low Values.
- Falsely low HbA1c (below 4%) may be observed in patients with clinical conditions that shorten erythrocyte life span or decrease mean erythrocyte age. HbA1c may not accurately reflect glycemic control when clinical conditions that affect erythrocyte survival are present.
- In cases of Interference of Hemoglobin variants in HbA1C, alternative methods (Fructosamine) estimation is recommended for Glycemic Control
 - HbF >25%
 - Homozygous Hemoglobinopathy.
(Hb Electrophoresis is recommended method for detection of Hemoglobinopathy)



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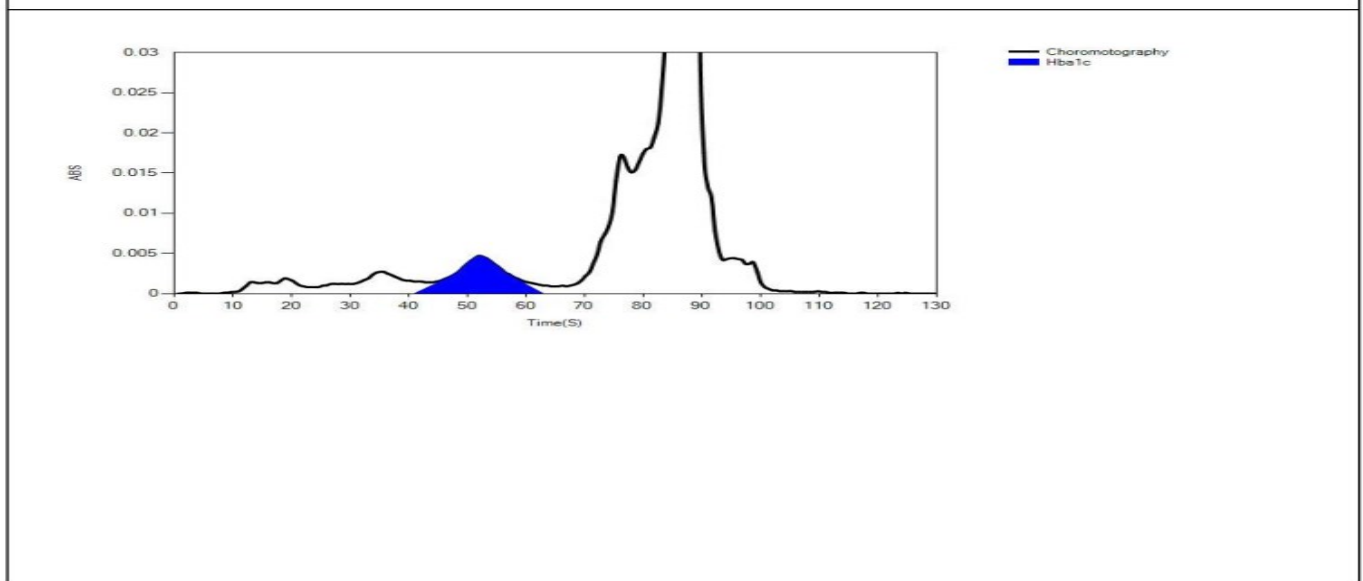
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DEPARTMENT OF BIOCHEMISTRY
AHC ABOVE 40Y FEMAL - SARAT BOSE ROAD

LIFOTRONIC Graph Report

Name :	Case :	Patient Type :	Test Date : 01/09/2025 16:49:18
Age :	Department :	Sample Type : Whole Blood EDTA	Sample Id : SBR250900011
Gender :			Total Area : 11889

Peak Name	Retention Time(s)	Absorbance	Area	Result (Area %)
HbA0	87	2512	10896	91.6
HbA1c	52	47	562	5.2
La1c	34	27	252	2.3
HbF	28	12	51	0.5
Hba1b	18	19	83	0.8
Hba1a	13	15	45	0.4



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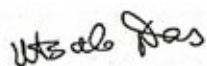
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DEPARTMENT OF BIOCHEMISTRY

AHC ABOVE 40Y FEMAL - SARAT BOSE ROAD

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DEPARTMENT OF BIOCHEMISTRY

AHC ABOVE 40Y FEMAL - SARAT BOSE ROAD

Test Name	Result	Unit	Bio. Ref. Interval	Method
LIVER FUNCTION TEST (LFT) WITH GGT , SERUM				
BILIRUBIN, TOTAL	0.84	mg/dL	ADULT 0.1 1.20~INFANT 1.2 12	(DIAZO)
BILIRUBIN CONJUGATED (DIRECT)	0.25	mg/dL	0.0-0.3	(DIAZO)
BILIRUBIN (INDIRECT)	0.59	mg/dL	ADULT 0 1.1~INFANT 0.6 10.5	Calculated
ASPARTATE AMINOTRANSFERASE (AST/SGOT)	46.73	U/L	0-45	(IFCC)
ALANINE AMINOTRANSFERASE (ALT/SGPT)	47.62	U/L	0-45	(IFCC)
AST (SGOT) / ALT (SGPT) RATIO	0.98	RATIO		Calculated
ALKALINE PHOSPHATASE	145.44	U/L	39 - 118	IFCC
PROTEIN, TOTAL	6.58	g/dL	6-8.3	Biuret
ALBUMIN	4.20	g/dL	3.5 - 5.5	Bromocresol green
GLOBULIN	2.38	g/dL	2.8-3.2	Calculated
A/G RATIO	1.76	RATIO	1.2-2.1	Calculated
GAMMA GLUTAMYL TRANSPETIDASE (GGT)	27.78	U/L	0-30	GLUPA - C

Comment:

LFT results reflect different aspects of the health of the liver, i.e., hepatocyte integrity (AST & ALT), synthesis and secretion of bile (Bilirubin, ALP), cholestasis (ALP, GGT), protein synthesis (Albumin) Common patterns seen:

1. Hepatocellular Injury: *AST – Elevated levels can be seen. However, it is not specific to liver and can be raised in cardiac and skeletal injuries.*ALT – Elevated levels indicate hepatocellular damage. It is considered to be most specific lab test for hepatocellular injury. Values also correlate well with increasing BMI. Disproportionate increase in AST, ALT compared with ALP. AST: ALT (ratio) – In case of hepatocellular injury AST: ALT > 1 In Alcoholic Liver Disease AST: ALT usually >2. This ratio is also seen to be increased in NAFLD, Wilsons's diseases, Cirrhosis, but the increase is usually not >2. Note- If both SGPT and SGOT are within reference range then AST:ALT (De Ritis ratio) does not have any clinical significance.

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DEPARTMENT OF BIOCHEMISTRY

AHC ABOVE 40Y FEMAL - SARAT BOSE ROAD

2. Cholestatic Pattern:*ALP – Disproportionate increase in ALP compared with AST, ALT. ALP elevation also seen in pregnancy, impacted by age and sex.*Bilirubin elevated- predominantly direct , To establish the hepatic origin correlation with elevated GGT helps.
3. Synthetic function impairment:*Albumin- Liver disease reduces albumin levels, Correlation with PT (Prothrombin Time) helps.
4. Associated tests for assessment of liver fibrosis - Fibrosis-4 and APRI Index.

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Test Name	Result	Unit	Bio. Ref. Interval	Method
UREA. , SERUM	18.66	mg/dL	13-45	Urease
BLOOD UREA NITROGEN , SERUM	9	mg/dL	6-21	Calculated

Test Name	Result	Unit	Bio. Ref. Interval	Method
CREATININE , SERUM	0.94	mg/dL	0.6-1.2	Jaffe

Test Name	Result	Unit	Bio. Ref. Interval	Method
URIC ACID , SERUM	3.40	mg/dL	2.5-6.8	Uricase

Comments:-

Uric acid is an end product of purine catabolism. Most uric acid is synthesised in the liver & from the intestine. Two thirds of uric acid is excreted by the kidneys.



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Test Name	Result	Unit	Bio. Ref. Interval	Method
LIPID PROFILE , SERUM				
TOTAL CHOLESTEROL	162.96	mg/dL	Desirable <200~Borderline 200- 239~High Risk:>240	Trinders
TRIGLYCERIDES.	54.94	mg/dL	35-170	GPO-Trinders
HDL CHOLESTEROL.	45.15	mg/dL	Low Risk>50~Moderate FSR 35-50~High Risk<30	
LDL CHOLESTEROL..	106.8	mg/dL	Optimal<100~Boderline High:130- 160~High:161- 190~Very High:>190	Calculated
NON - HDL CHOLESTEROL	117.81	mg/dl	<130	Calculated
VLDL CHOLESTEROL	11.0	mg/dL	<40	Calculated
CHOL / HDL RATIO	3.61		0-5	Calculated
LDL / HDL RATIO	2.37		0-3.55	Calculated

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DEPARTMENT OF IMMUNOLOGY

AHC ABOVE 40Y FEMAL - SARAT BOSE ROAD

Test Name	Result	Unit	Bio. Ref. Interval	Method
THYROID STIMULATING HORMONE (TSH) , SERUM	2.7	µIU/mL	0.25-5	ELISA

Comment:

TSH is a glycoprotein hormone secreted by the anterior pituitary. TSH is a labile hormone & is secreted in a pulsatile manner throughout the day and is subject to several non-thyroidal pituitary influences. Significant variations in TSH can occur with circadian rhythm, hormonal status, stress, sleep deprivation, caloric intake, medication & circulating antibodies.

It is important to confirm any TSH abnormality in a fresh specimen drawn after ~ 3 weeks before assigning a diagnosis, as the cause of an isolated TSH abnormality.

For pregnant females	Bio Ref Range for TSH in uIU/ml (As per American Thyroid Association)
First trimester	0.1 - 2.5
Second trimester	0.2 – 3.0
Third trimester	0.3 – 3.0

Test Name	Result	Unit	Bio. Ref. Interval	Method
VITAMIN D (25 - OH VITAMIN D) , SERUM	21.3	ng/mL	30-100	ELISA

Comment:

BIOLOGICAL REFERENCE RANGES

VITAMIN D STATUS	VITAMIN D 25 HYDROXY (ng/mL)
DEFICIENCY	<10
INSUFFICIENCY	10 – 30
SUFFICIENCY	30 – 100
TOXICITY	>100

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DEPARTMENT OF IMMUNOLOGY

AHC ABOVE 40Y FEMAL - SARAT BOSE ROAD

The biological function of Vitamin D is to maintain normal levels of calcium and phosphorus absorption. 25-Hydroxy vitamin D is the storage form of vitamin D. Vitamin D assists in maintaining bone health by facilitating calcium absorption. Vitamin D deficiency can also cause osteomalacia, which frequently affects elderly patients.

Vitamin D Total levels are composed of two components namely 25-Hydroxy Vitamin D2 and 25-Hydroxy Vitamin D3 both of which are converted into active forms. Vitamin D2 level corresponds with the exogenous dietary intake of Vitamin D rich foods as well as supplements. Vitamin D3 level corresponds with endogenous production as well as exogenous diet and supplements. Vitamin D from sunshine on the skin or from dietary intake is converted predominantly by the liver into 25-hydroxy vitamin D, which has a long half-life and is stored in the adipose tissue. The metabolically active form of vitamin D, 1,25-di-hydroxy vitamin D, which has a short life, is then synthesized in the kidney as needed from circulating 25-hydroxy vitamin D. The reference interval of greater than 30 ng/mL is a target value established by the Endocrine Society.

Decreased Levels:- Inadequate exposure to sunlight, Dietary deficiency, Vitamin D malabsorption, Severe Hepatocellular disease., Drugs like Anticonvulsants, Nephrotic syndrome.

Increased levels:- Vitamin D intoxication.

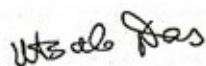
Test Name	Result	Unit	Bio. Ref. Interval	Method
VITAMIN B12 , SERUM	304	pg/mL	200-835	ELISA

Comment:

Population based data reflecting exact scenario of vitamin B12 levels in Indian population is still evolving, however, different studies reporting a deficiency in adults, pregnant women and children ranging from 16% to 77% with average of about 47%. This high incidence is attributed to vegetarian food habits of large majority of Indian population.

Vitamin B12 deficiency frequently causes macrocytic anemia, glossitis, peripheral neuropathy, weakness, hyperreflexia, ataxia, loss of proprioception, poor coordination, and affective behavioral changes. A significant increase in RBC MCV may be an important indicator of vitamin B12 deficiency. B12 levels in the range of 150 to 190 pg/ml may not be associated with any clinical manifestations, while B12 levels below 100 pg/ml are often associated with clinical symptoms. However, for an individual based on other co-morbid conditions or other nutritional deficiency (especially folate) the manifestations can vary accordingly.

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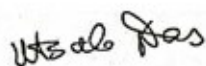
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If clinical symptoms suggest deficiency, measurement of active vitamin B12, MMA and homocysteine should be considered as further workup.



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DEPARTMENT OF CLINICAL PATHOLOGY
AHC ABOVE 40Y FEMAL - SARAT BOSE ROAD

Test Name	Result	Unit	Bio. Ref. Interval	Method
COMPLETE URINE EXAMINATION (CUE),, URINE				
PHYSICAL EXAMINATION				
VOLUME	40	ml		Volumetric
COLOUR	PALE STRAW		Clear To Pale Yellow	Macroscopy
APPEARANCE	SLIGHT HAZY			Macroscopy
CLINICAL EXAMINATION				
SP. GRAVITY	1.005		1.005-1.03	Dipstick
pH	6.0			Dipstick
URINE PROTEIN	NEGATIVE		NEGATIVE	Dipstick
GLUCOSE	NEGATIVE		Negative	Dipstick
URINE BILIRUBIN	NEGATIVE		NEGATIVE	Dipstick
URINE KETONES	NEGATIVE		NEGATIVE	Dipstick
UROBILINOGEN	Normal		NORMAL	Dipstick
OCCULT BLOOD	NEGATIVE		NEGATIVE	DIPSTICK
NITRITE	NEGATIVE		NEGATIVE	Dipstick
LEUCOCYTE ESTERASE	NEGATIVE		NEGATIVE	Dipstick
MICROSCOPIC EXAMINATION				
PUS CELLS	2-3	/hpf	0-5	Microscopy
RBC	NIL	/hpf	0-2	Microscopy
EPITHELIAL CELLS	3-4	/hpf	<10	Microscopy
CASTS	NIL			Microscopy
CRYSTALS	NOT FOUND		ABSENT	Microscopy
BACTERIA	PLENTY		Absent	Microscopy

***** End Of Report *****

Result/s to Follow:

Page 16 of 17

Utsab Das

Dr Utsab Das
MBBS, MD(PATH)
CONSULTANT

SIN No:SBR250900015



Patient Name : Mrs.ASMITA DASGUPTA
Age/Gender : 51 Y 7 M 15 D/F
UHID/MR No : FSBR.0000018456
Visit ID : FSBROPV29438
Ref Doctor : Self
Emp/Auth/TPA ID : 123

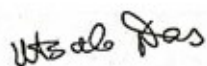
Collected : 01/Sep/2025 09:34AM
Received : 01/Sep/2025 09:55AM
Reported : 01/Sep/2025 07:27PM
Status : Final Report
Sponsor Name : SGD 369 HCL

DEPARTMENT OF CLINICAL PATHOLOGY

AHC ABOVE 40Y FEMAL - SARAT BOSE ROAD

STOOL ROUTINE EXAMINATION

Page 17 of 17



Dr Utsab Das
MBBS, MD(PATH)
CONSULTANT

SIN No:SBR250900015



Apollo Clinic

CONSENT FORM

Patient Name : ASHITA DASGUPTA Age : 51/F

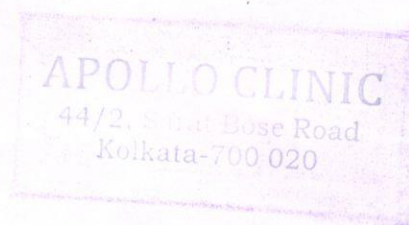
UHID Number : Company Name :

I Mr./Mrs./Ms ☒ Asmita Dasgupta Employee of :

(Company) Want to inform your that I am not interested in getting : EYE, Dental,

Tests done which is a part of my routine health check package. She come tom
3/9/25
And : I claim the above statement in my full consciousness. for mammography

Patient Signature : For Asmita Dasgupta Date : 01/09/25





ASMITA DASGUPTA / 51 Yrs / F / 158 Cms / 56 Kg

Refd By : SELF Examined By :

Pain Angina / Non-Hypercholesterolemia / Non-Diabetic / Negative Estrogen / Non-Athlete

Time	Duration	Speed(mph)	Elevation	METs	Rate	% THR	BP	RPP	PVC	Comments
00:09	0:09	00.0	00.0	01.0	079	47 %	150/90	118	00	
00:15	0:06	00.0	00.0	01.0	079	47 %	150/90	118	00	
00:19	0:04	00.0	00.0	01.0	079	47 %	150/90	118	00	
00:23	0:04	01.0	00.0	01.0	083	49 %	150/90	124	00	
00:52	0:29	01.0	00.0	01.0	102	60 %	150/90	153	00	
03:17	2:25	01.7	10.0	04.0	165	98 %	170/90	280	00	
03:47	0:30	01.1	00.0	04.0	165	98 %	170/90	280	00	
04:17	1:00	01.1	00.0	04.0	164	97 %	170/90	278	00	
05:17	2:00	00.0	00.0	04.0	133	79 %	170/90	226	00	
06:17	3:00	00.0	00.0	04.0	104	62 %	170/90	176	00	
07:17	4:00	00.0	00.0	04.0	092	54 %	160/90	147	00	
08:17	5:00	00.0	00.0	04.0	085	50 %	160/90	136	00	
09:17	6:00	00.0	00.0	04.0	105	62 %	160/90	168	00	
10:17	7:00	00.0	00.0	04.0	097	57 %	160/90	155	00	
10:35	7:18	00.0	00.0	04.0	092	54 %	150/90	138	00	

IGS :

Exercise Time

Initial HR (ExStrt)

Initial BP (ExStrt)

Max WorkLoad Attained

Max ST Dep Lead & Avg ST Value : II & -2.0 mm in ExStart

Max Treadmill Score

Test End Reasons

RT :

: 02:25

: 102 bpm 60% of Target 169

: 150/90 (mm/Hg)

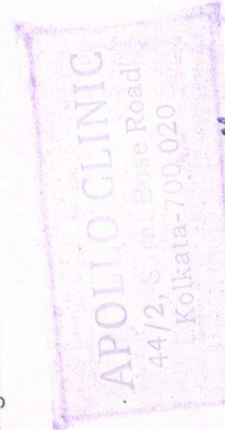
: 4 Poor response to induced stress

: 02.4

: Test Complete, Heart Rate Achieved

Max HR Attained 165 bpm 98% of Target 169

Max BP Attained 170/90 (mm/Hg)



Dr. Anup Shyamal

Asmita Dasgupta

Doctor : DR. ANUP SHYAMAL MBBS, MD

ID: 3

ASMITA DASGUPTA

Female 51Years

Req. No. :

Asmita Dasgupta

01-09-2025

10:12:11 AM

HR : 60 bpm

P : 84 ms

PR : 136 ms

QRS : 75 ms

QT/QTcBz : 403/404 ms

P/QRS/T : 59/24/36 °

RV5/SV1 : 1.201/1.206 mV

Diagnosis Information:

Sinus Rhythm

Larged PtfV1

Poor r Wave Progression(V3)

Ref-Phys. :

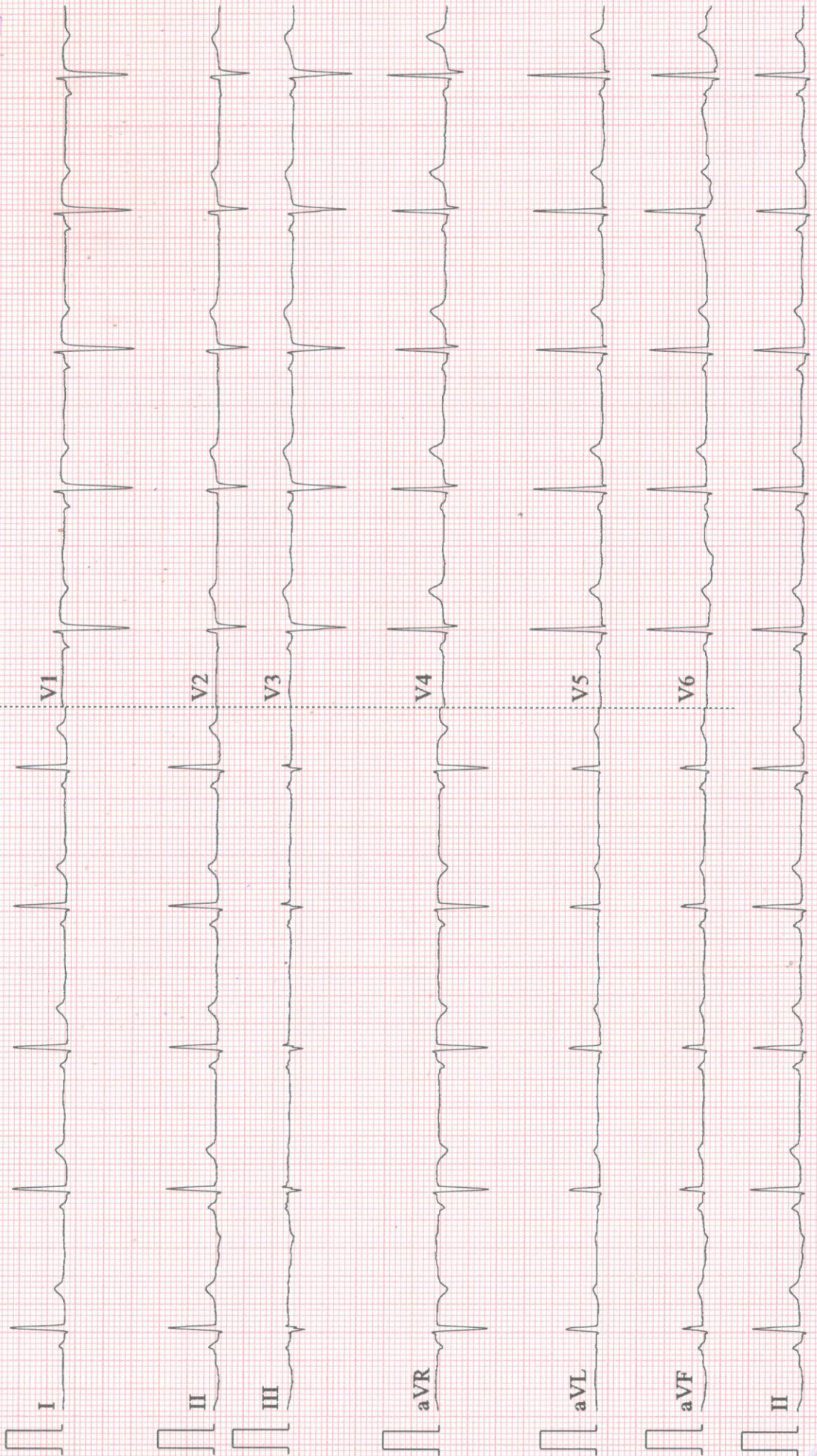
Report Confirmed by:

CARDIART

Dr. Subrata

APOLLO CLINIC

44/2, Sarat Bose Road
Kolkata-700020



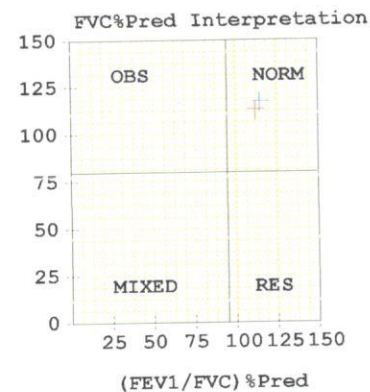
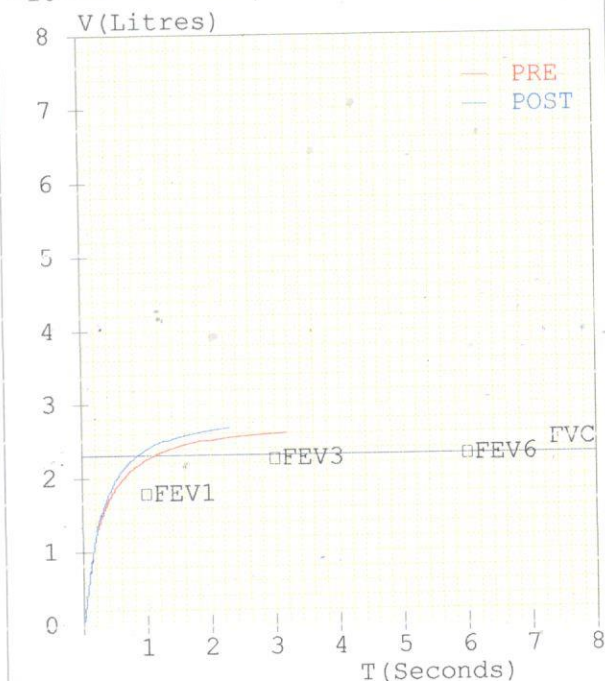
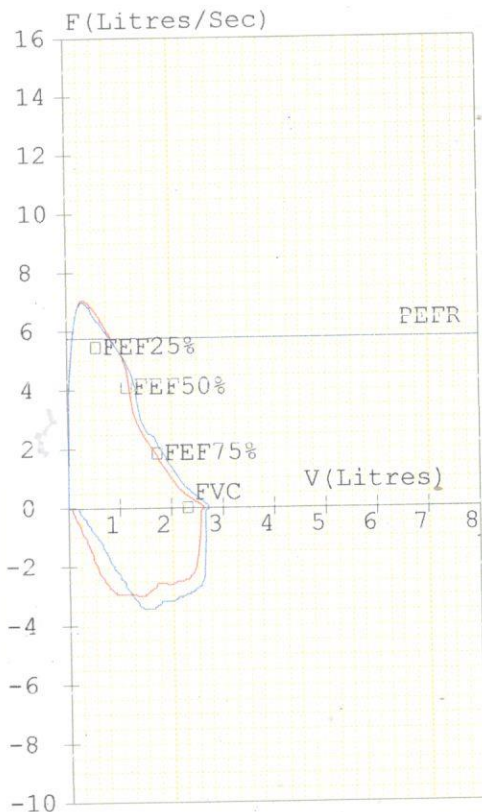
APOLLO CLINIC SARAT BOSE ROAD

44/2, SARAT BOSE ROAD KOLKATA-700020

Patient: ASMITA DASGUPTA
Ref. By: SELF
Ref. Eqns: RECORDERS
Date : 01-Sep-2025 12:58 PM

Age : 51 Yrs
Height : 158 Cms
Weight : 56 Kgs
ID : 18456

Gender : Female
Smoker : No
Eth. Corr: 100
Temp :



FVC Results

Parameter		Pred	M.Pre	%Pred	M.Post	%Pred	%Imp
FVC	(L)	02.31	02.62	113	02.71	117	+03
FEV1	(L)	01.77	02.25	127	02.39	135	+06
FEV1/FVC	(%)	76.62	85.88	112	88.19	115	+03
FEF25-75	(L/s)	02.22	02.64	119	02.92	132	+11
PEFR	(L/s)	05.76	06.98	121	06.94	120	-01
FIVC	(L)	-----	02.54	---	02.51	---	-01
FEV.5	(L)	-----	01.85	---	01.94	---	+05
FEV3	(L)	02.24	02.61	117	02.71	121	+04
PIFR	(L/s)	-----	02.97	---	03.43	---	+15
FEF75-85	(L/s)	-----	00.71	---	00.89	---	+25
FEF.2-1.2	(L/s)	04.19	05.84	139	05.84	139	---
FEF 25%	(L/s)	05.45	06.32	116	06.08	112	-04
FEF 50%	(L/s)	04.08	03.01	074	03.58	088	+19
FEF 75%	(L/s)	01.86	01.05	056	01.28	069	+22
FEV.5/FVC	(%)	-----	70.61	---	71.59	---	+01
FEV3/FVC	(%)	96.97	99.62	103	100.00	103	---
FET	(Sec)	-----	03.19	---	02.53	---	---
ExptTime	(Sec)	-----	00.04	---	00.04	---	---
Lung Age	(Yrs)	051	037	073	033	065	-11
FEV6	(L)	02.31	-----	---	-----	---	---
FIF25%	(L/s)	-----	02.51	---	02.93	---	+17
FIF50%	(L/s)	-----	02.95	---	03.39	---	+15
FIF75%	(L/s)	-----	02.67	---	02.41	---	-10

APOLLO CLINIC
44/2, Sarat Bose Road
Kolkata-700 020

DR. RANAJAY CHANDA

MBBS, DCCP(LUCKNOW), FCCP(USA)
FELLOWSHIP IN PULMONOLOGY (HOSPITAL KOLKATA)
EUROPEAN DIPLOMA IN RESPIRATORY MEDICINE (ERS)
REG NO. - 64557WIMC

Pre Medication Report Indicates

Spirometry within normal limits as (FEV1/FVC)%Pred >95 and FVC%Pred >80

Post Medication Report Indicates

Spirometry within normal limits as (FEV1/FVC)%Pred >95 and FVC%Pred >80

Patient ID: 012509010181	Primary Sample Collection Time: 01/Sep/2025 15:16:17
Name: Mr. ASMITA DASGUPTA	Collection Date/Time: 01/Sep/2025 03:23PM
Age/Gender: 51 Y/Male	Received Date/Time: 01/Sep/2025 03:56PM
Referred By: Self	Approved Date/Time: 01/Sep/2025 05:50PM
	Print Date/Time: 02/Sep/2025 12:39PM

DEPARTMENT OF CLINICAL PATHOLOGY

Test Description	Observed Value	Unit	Method	Biological Ref. Interval
STOOL FOR ROUTINE EXAMINATION :				
PHYSICAL EXAMINATION:				
COLOUR	BROWNISH			Whitish
CONSISTENCY	SOLID			
Mucus	ABSENT			Absent
VISIBLE BLOOD	ABSENT			
CHEMICAL EXAMINATION :-				
REACTION / pH	ACIDIC			5.80-7.50
STOOL REDUCING SUBSTANCE	ABSENT		Chemical	Negative
OCCULT BLOOD	NEGATIVE			
MICROSCOPIC EXAMINATION:-				
PUS CELLS	2-3	/hpf		NIL
RBC	NOT FOUND	/HPF		
MACROPHAGE	NOT FOUND	%		
OVA	NOT FOUND	MICROSCOPIC		NIL
CYST	NOT FOUND	MICROSCOPIC		NIL
TROPHOZOITES	NOT FOUND			
Larvae	NOT FOUND			
PARASITES :	NOT FOUND		MICROSCOPIC	Not Seen
VEG. CELLS	PRESENT			

Disclaimer: The test result mentioned here should be interpreted in view of clinical condition of the patient. In case of any clinical suspicion regarding any parameter, repeat test with fresh sample essential to conclude.

*** End Of Report ***

APOLLO CLINIC
44/2, Sarat Bose Road,
Kolkata-700 020

Dr. Tushar Kanti Das

MD(Path)

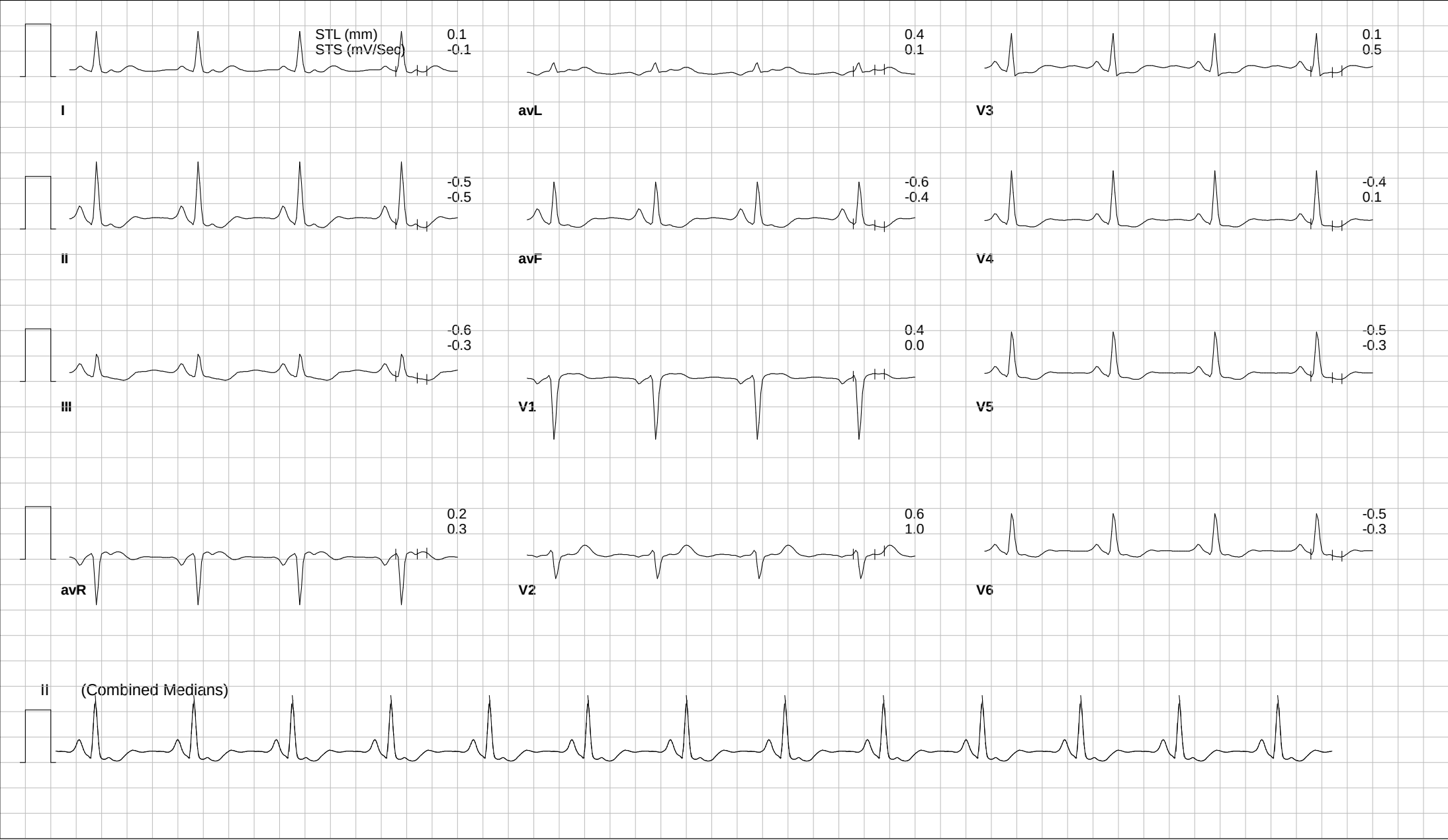
TO BOOK AN APPOINTMENT

9038623111

9038606999

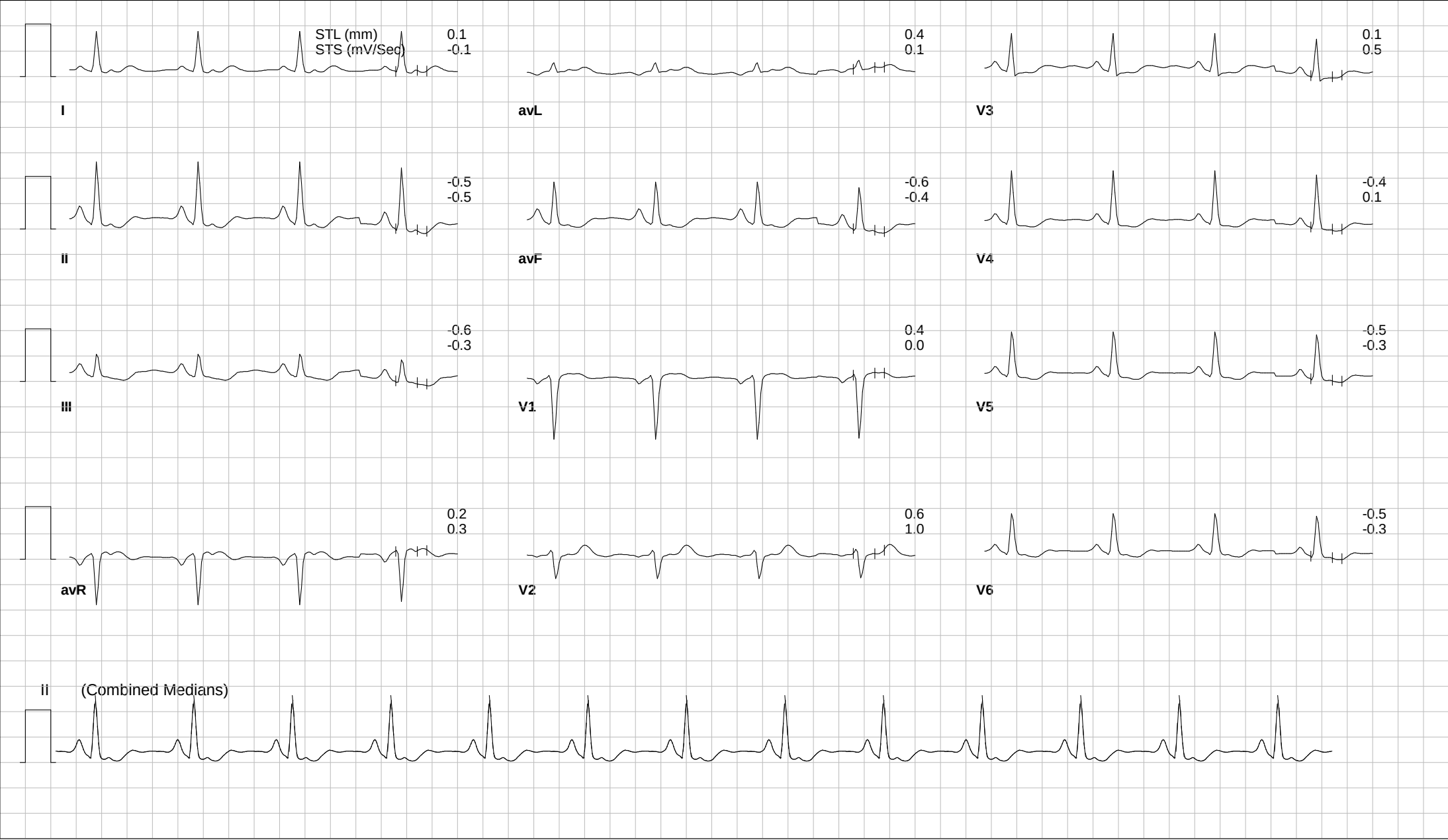


Date: 01 - 09 - 2025 METs : 1.0 HR : 79 Target HR : 47% of 169 BP : 150/90 Post J @70mSec ExTime: 00:00 Speed: 0.0 mph Grade : 00.00 % 25 mm/Sec. 1.0 Cm/mV



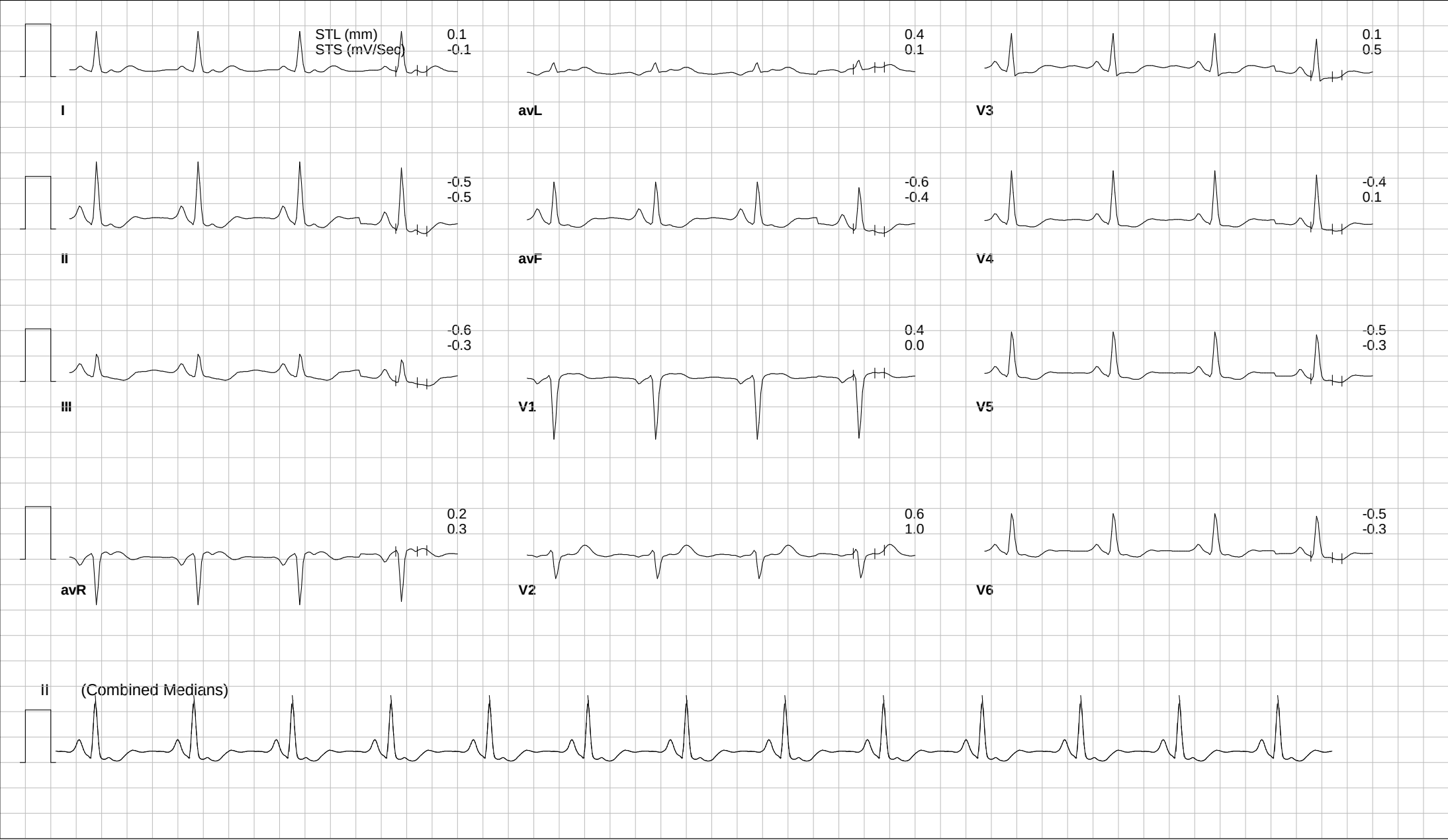


Date: 01 - 09 - 2025 METs : 1.0 HR : 79 Target HR : 47% of 169 BP : 150/90 Post J @80mSec ExTime: 00:00 Speed: 0.0 mph Grade : 00.00 % 25 mm/Sec. 1.0 Cm/mV



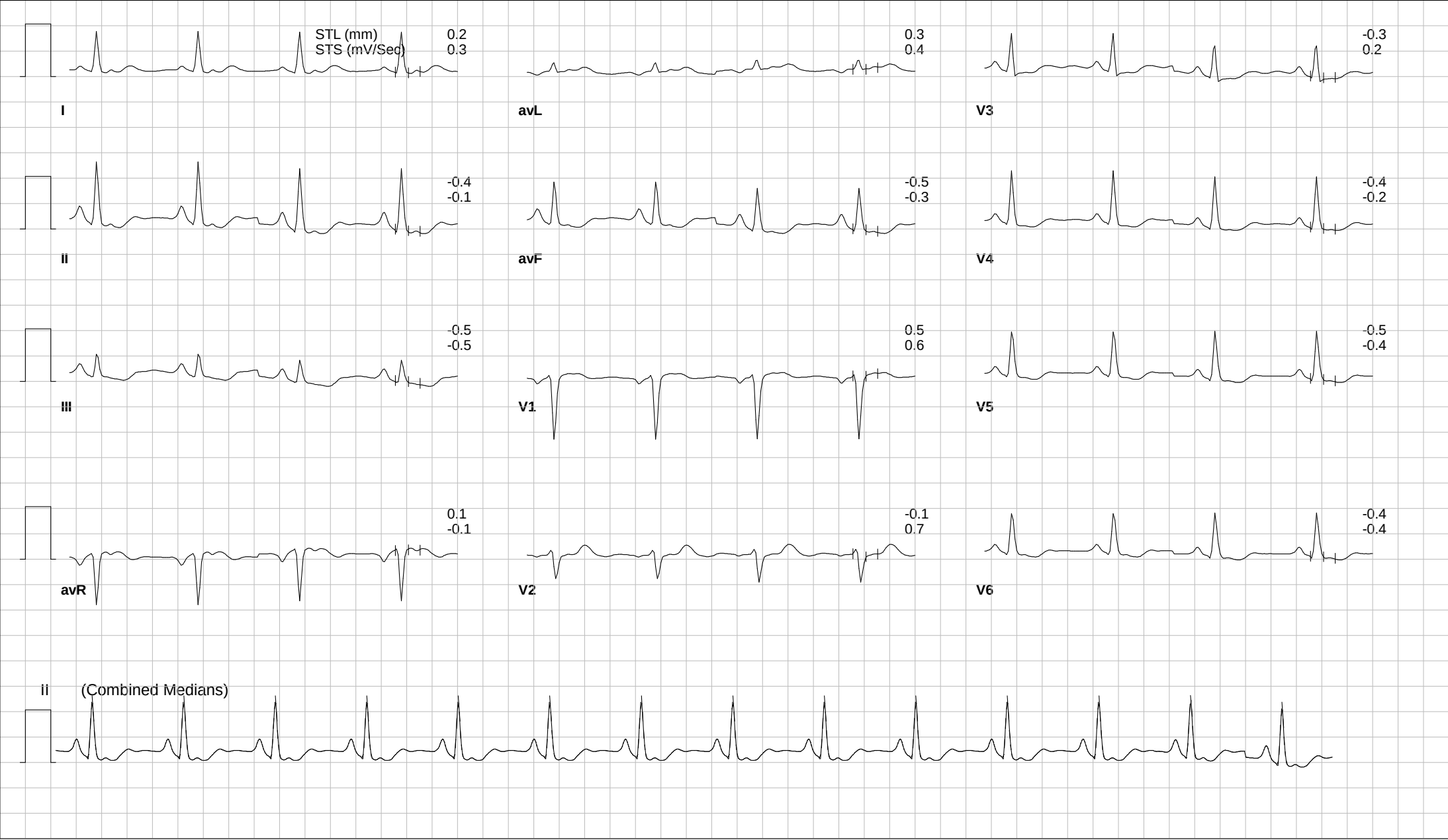


Date: 01 - 09 - 2025 METs : 1.0 HR : 79 Target HR : 47% of 169 BP : 150/90 Post J @70mSec ExTime: 00:00 Speed: 0.0 mph Grade : 00.00 % 25 mm/Sec. 1.0 Cm/mV



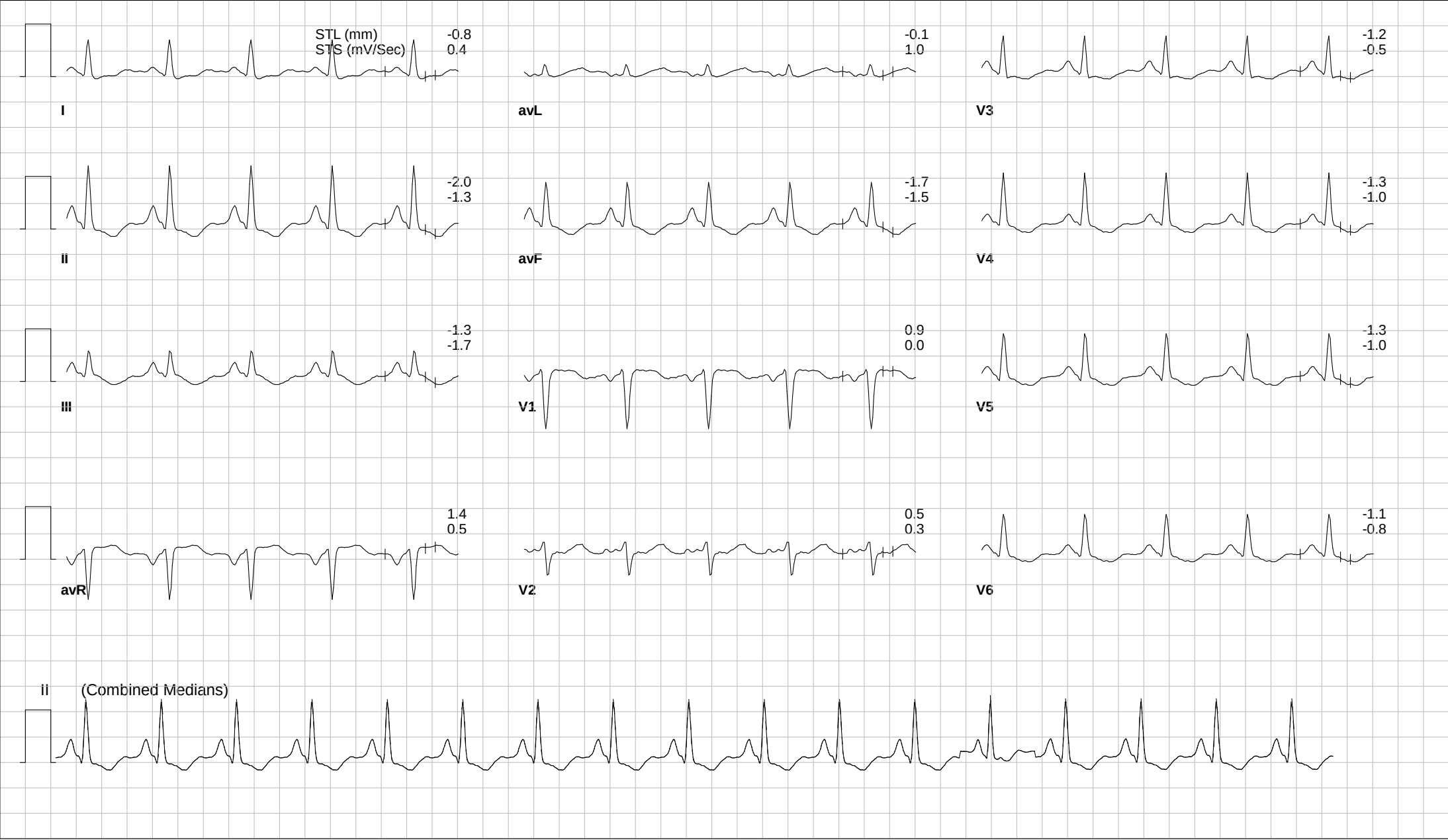


Date: 01 - 09 - 2025 METs : 1.0 HR : 83 Target HR : 49% of 169 BP : 150/90 Post J @80mSec ExTime: 00:00 Speed: 1.0 mph Grade : 00.00 % 25 mm/Sec. 1.0 Cm/mV



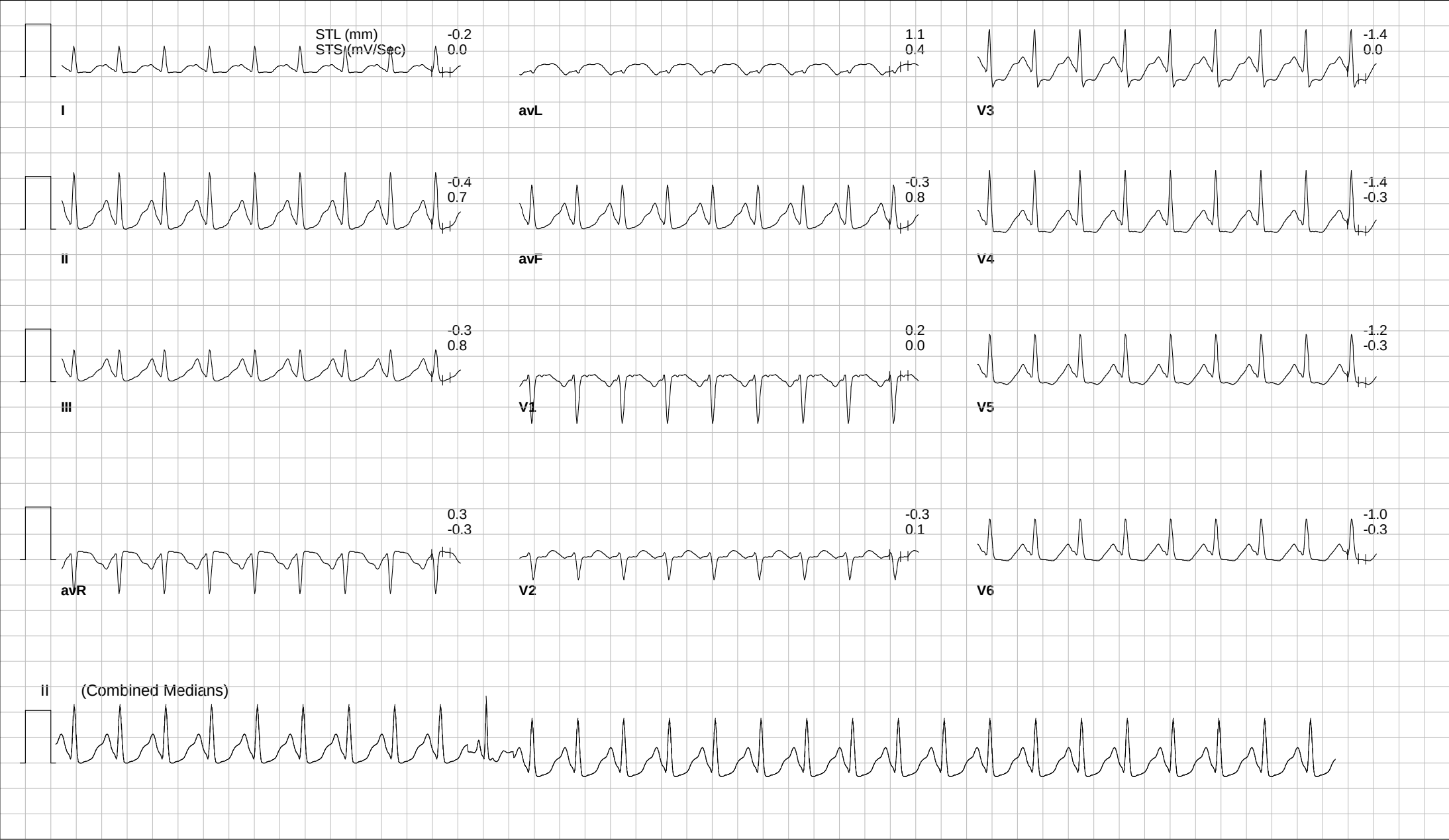


Date: 01 - 09 - 2025 METs : 1.0 HR : 102 Target HR : 60% of 169 BP : 150/90 Post J @70mSec ExTime: 00:00 Speed: 1.0 mph Grade : 00.00 % 25 mm/Sec. 1.0 Cm/mV



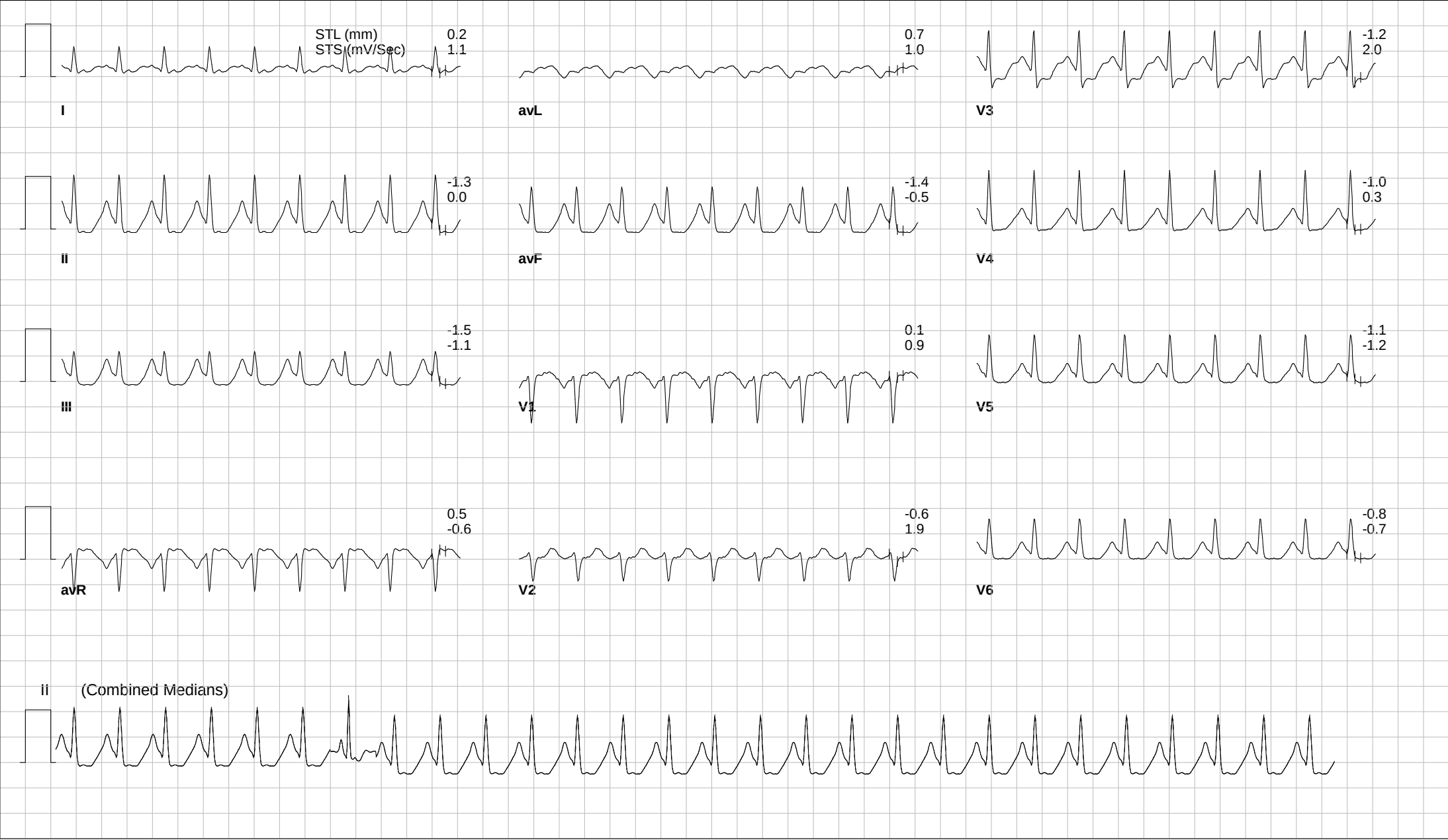


Date: 01 - 09 - 2025 METs : 4.0 HR : 165 Target HR : 98% of 169 BP : 170/90 Post J @60mSec ExTime: 02:25 Speed: 1.7 mph Grade : 10.00 % 25 mm/Sec. 1.0 Cm/mV



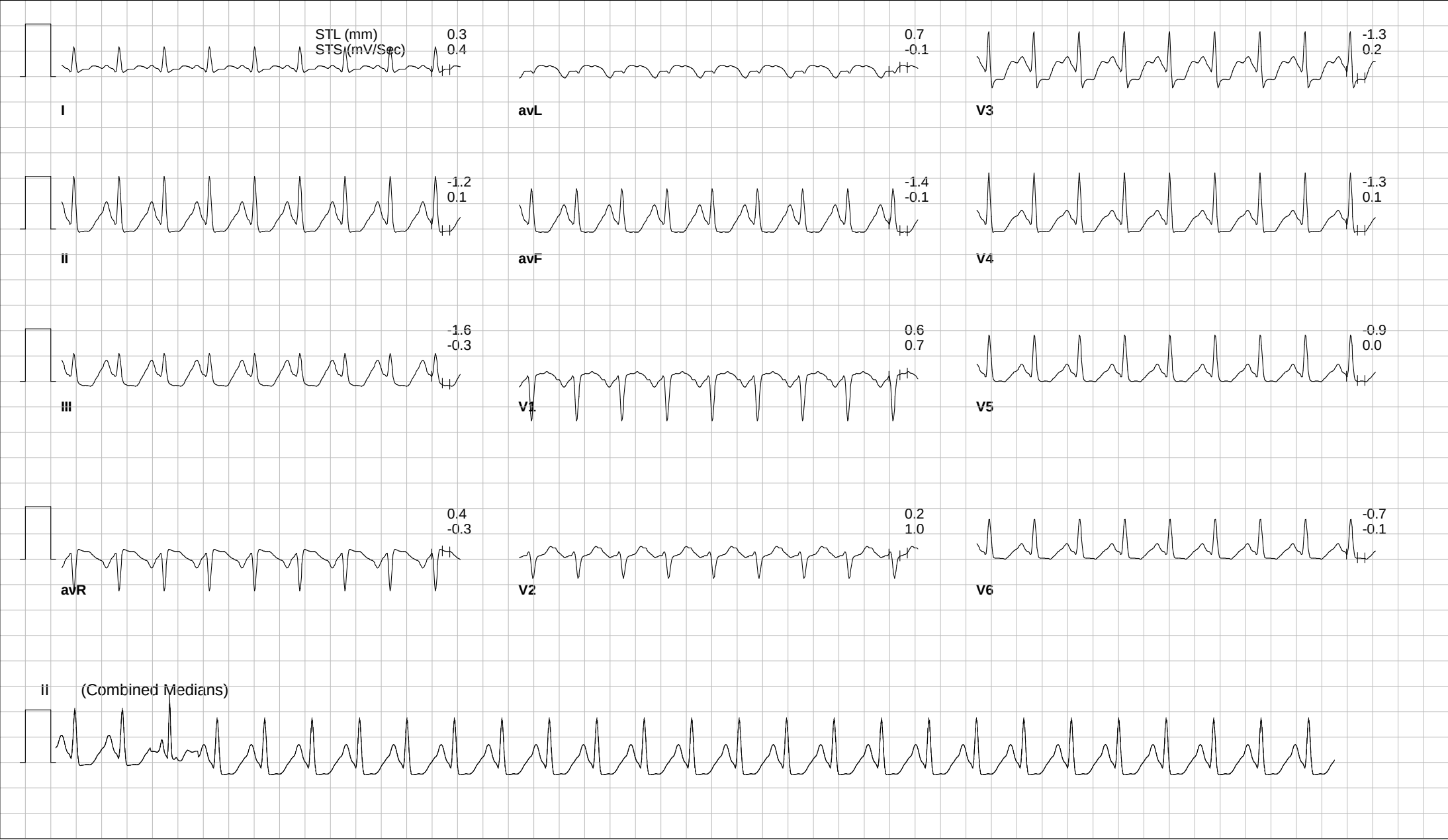


Date: 01 - 09 - 2025 METs : 4.0 HR : 165 Target HR : 98% of 169 BP : 170/90 Post J @40mSec ExTime: 02:25 Speed: 1.1 mph Grade : 00.00 % 25 mm/Sec. 1.0 Cm/mV



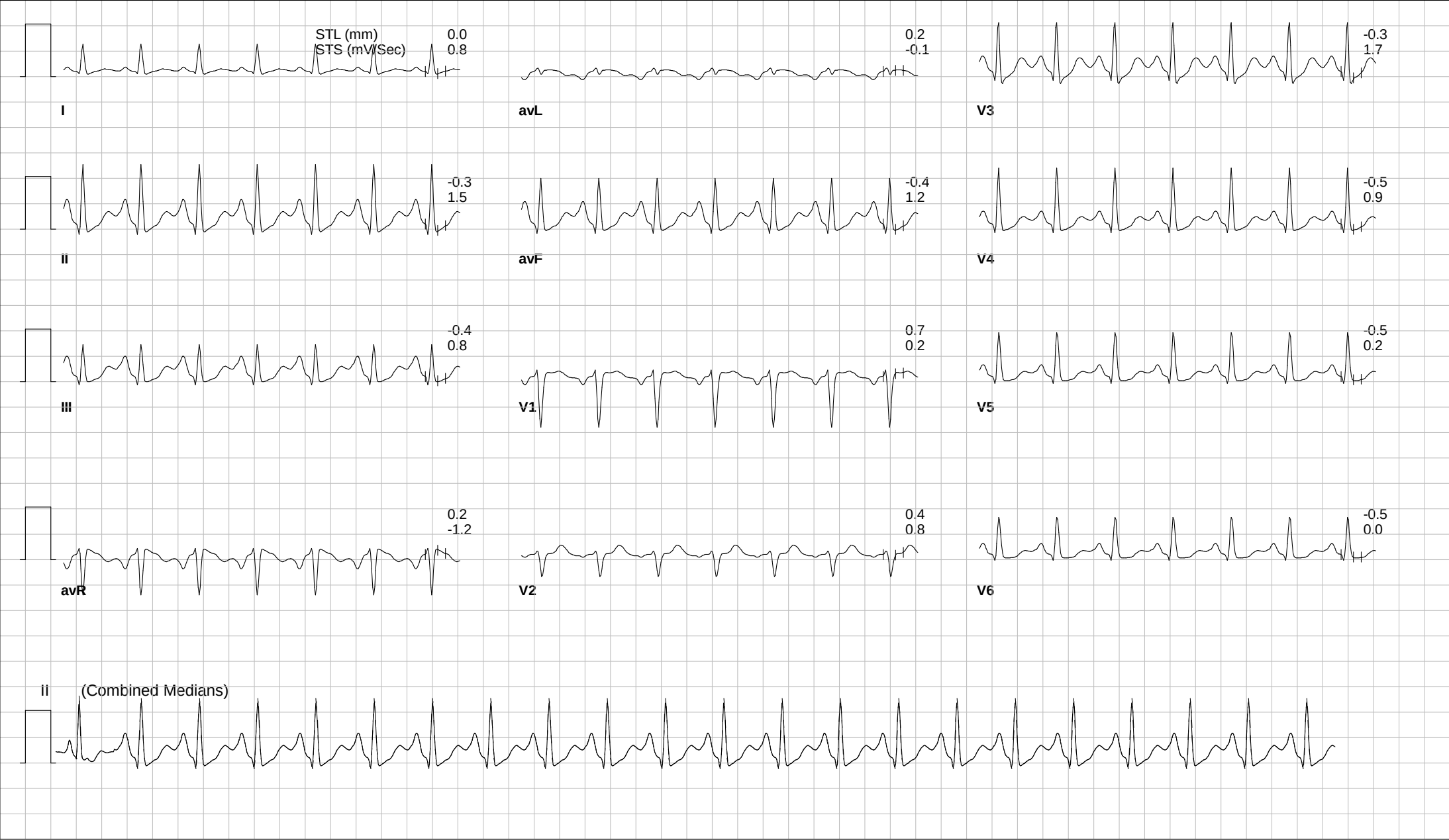


Date: 01 - 09 - 2025 METs : 4.0 HR : 164 Target HR : 97% of 169 BP : 170/90 Post J @60mSec ExTime: 02:25 Speed: 1.1 mph Grade : 00.00 % 25 mm/Sec. 1.0 Cm/mV



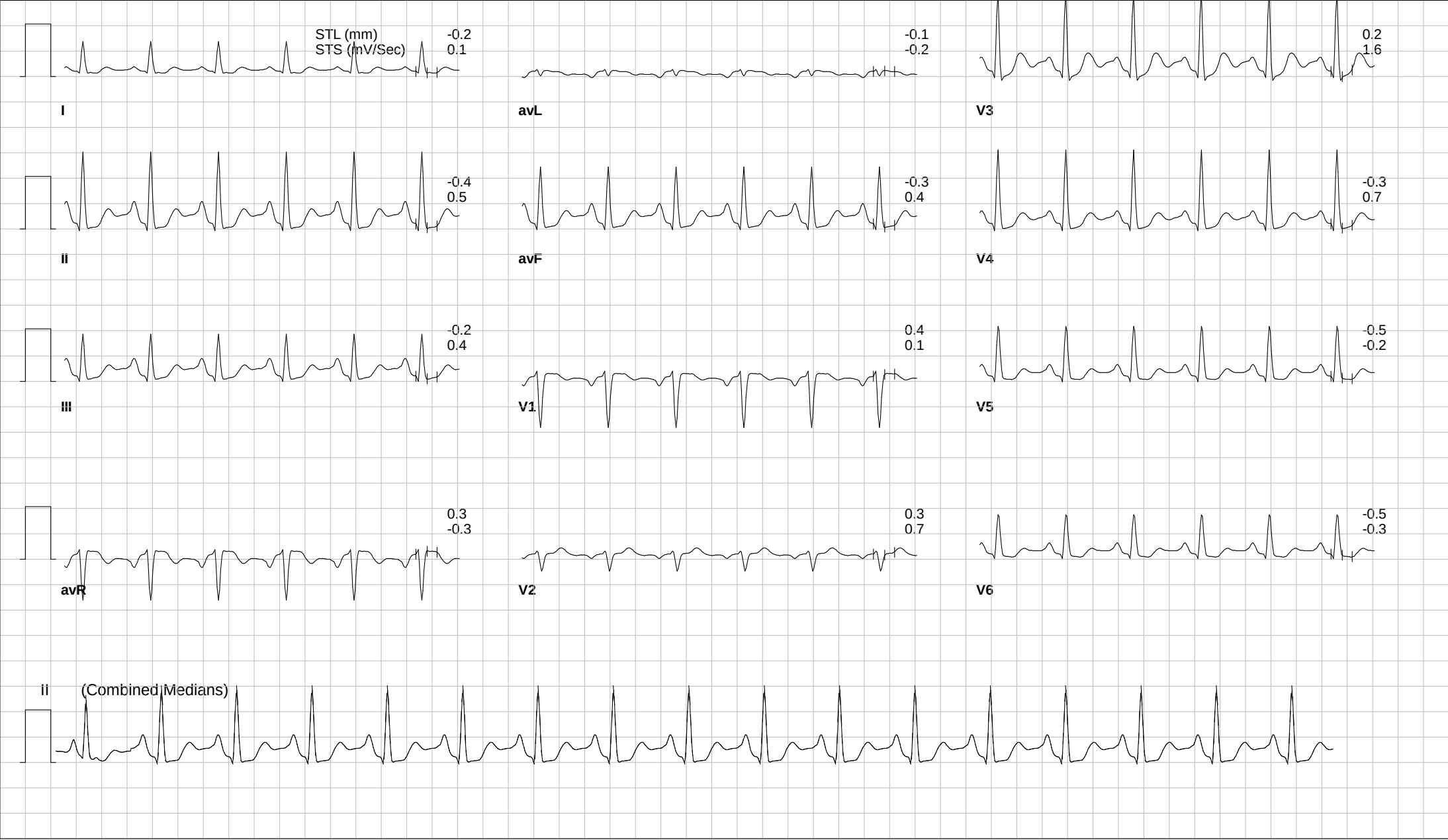


Date: 01 - 09 - 2025 METs : 4.0 HR : 133 Target HR : 79% of 169 BP : 170/90 Post J @60mSec ExTime: 02:25 Speed: 0.0 mph Grade : 00.00 % 25 mm/Sec. 1.0 Cm/mV



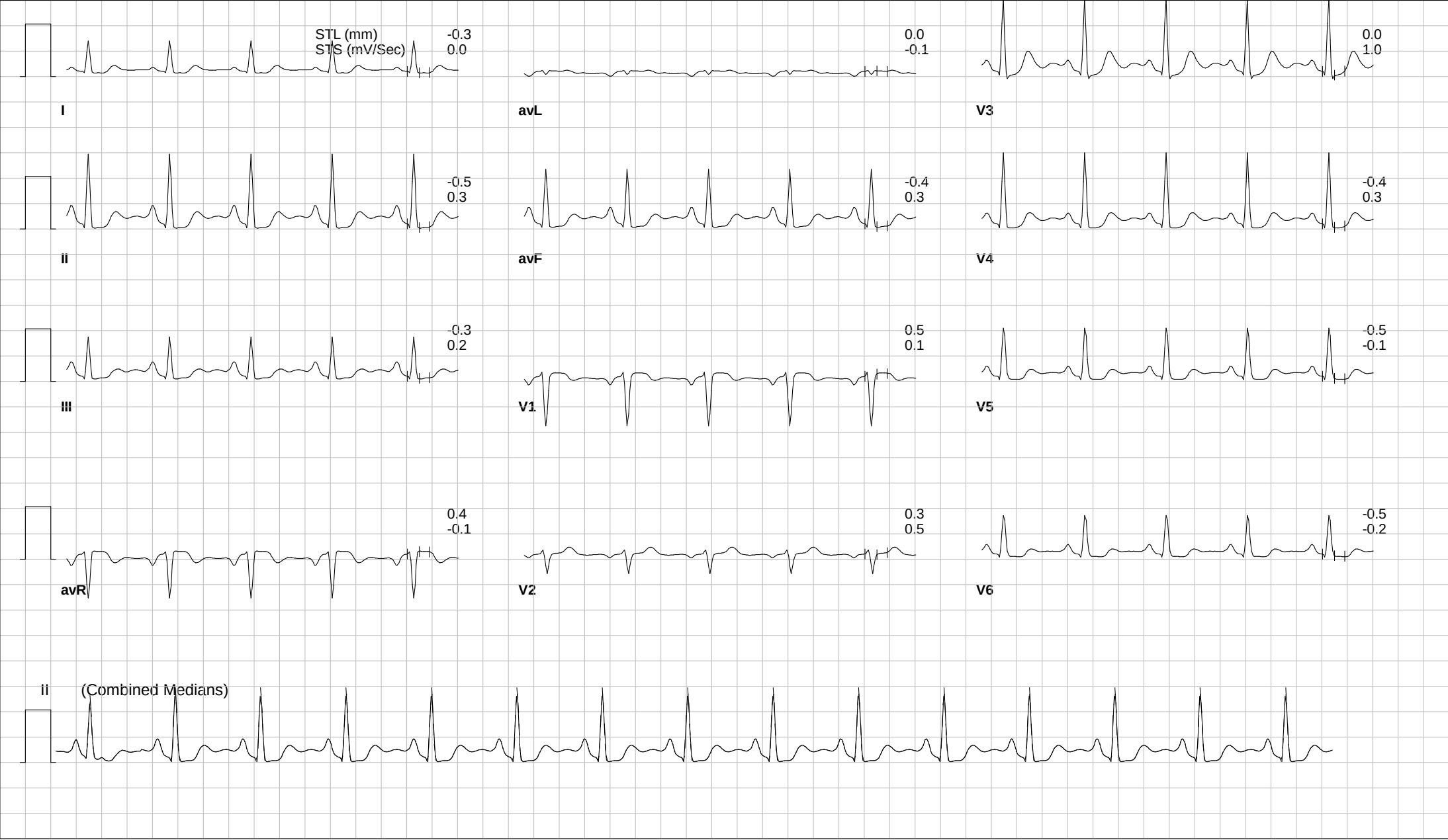


Date: 01 - 09 - 2025 METs : 4.0 HR : 104 Target HR : 62% of 169 BP : 170/90 Post J @80mSec ExTime: 02:25 Speed: 0.0 mph Grade : 00.00 % 25 mm/Sec. 1.0 Cm/mV



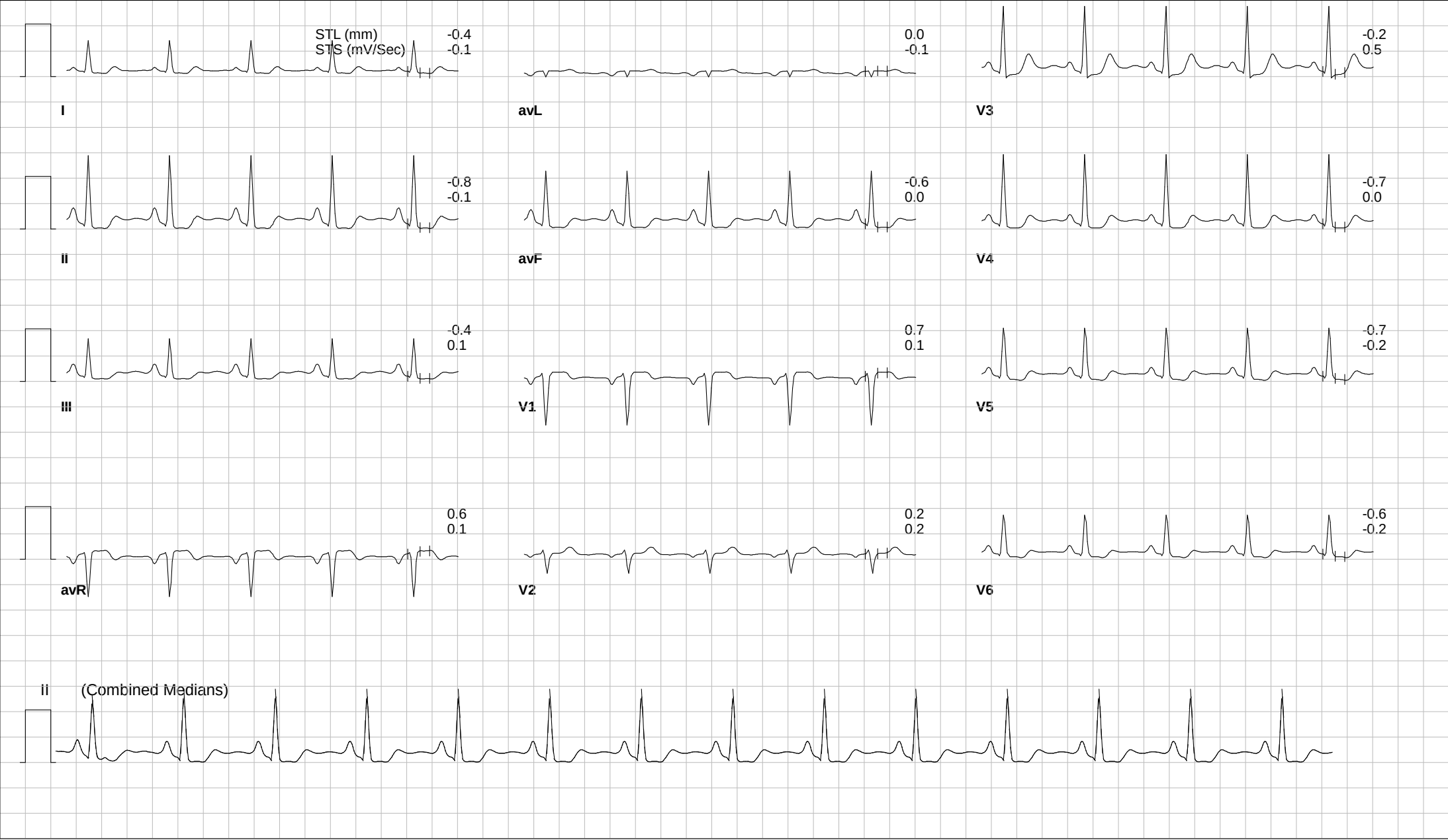


Date: 01 - 09 - 2025 METs : 4.0 HR : 92 Target HR : 54% of 169 BP : 160/90 Post J @80mSec ExTime: 02:25 Speed: 0.0 mph Grade : 00.00 % 25 mm/Sec. 1.0 Cm/mV



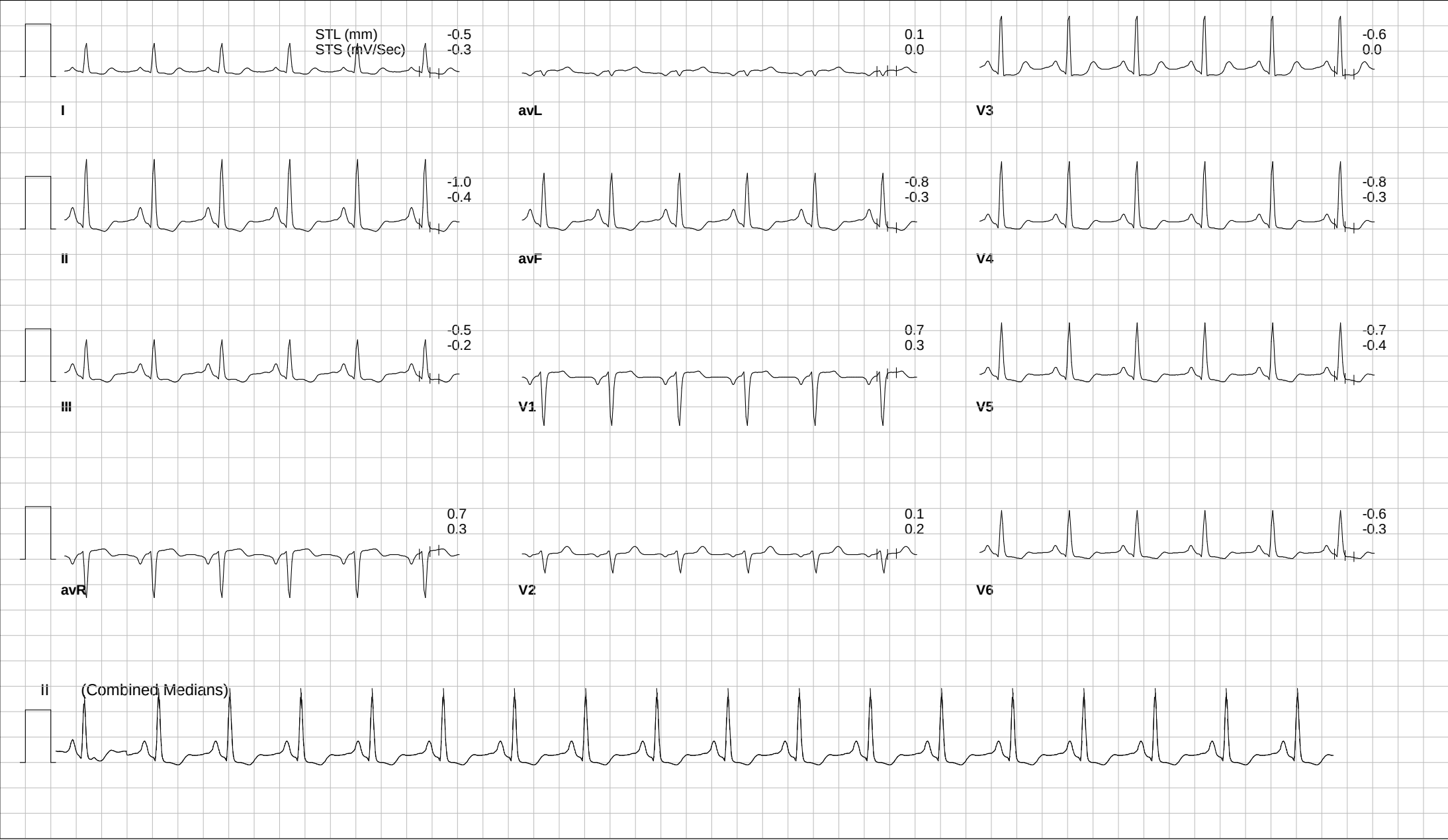


Date: 01 - 09 - 2025 METs : 4.0 HR : 85 Target HR : 50% of 169 BP : 160/90 Post J @80mSec ExTime: 02:25 Speed: 0.0 mph Grade : 00.00 % 25 mm/Sec. 1.0 Cm/mV



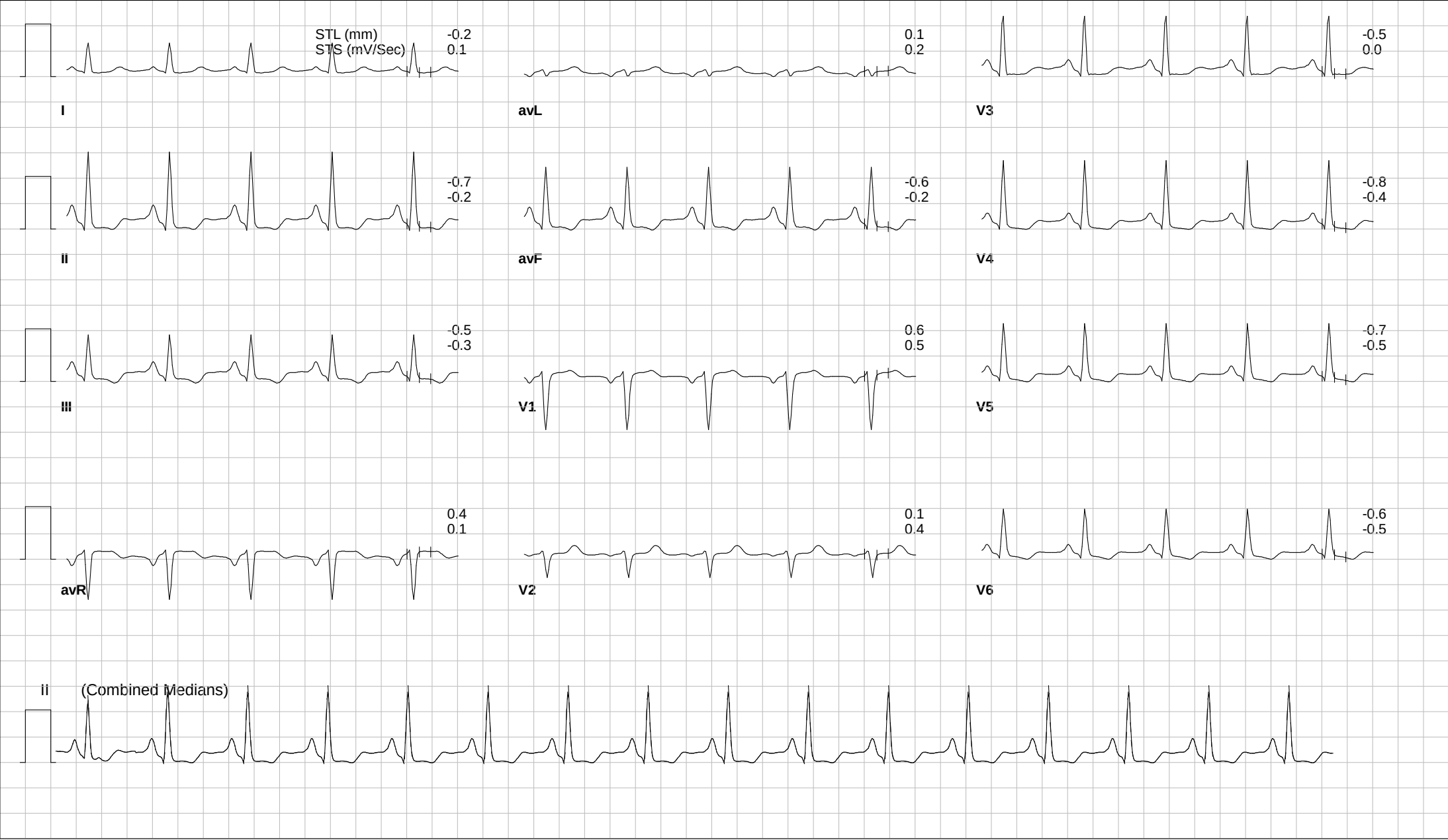


Date: 01 - 09 - 2025 METs : 4.0 HR : 105 Target HR : 62% of 169 BP : 160/90 Post J @80mSec ExTime: 02:25 Speed: 0.0 mph Grade : 00.00 % 25 mm/Sec. 1.0 Cm/mV



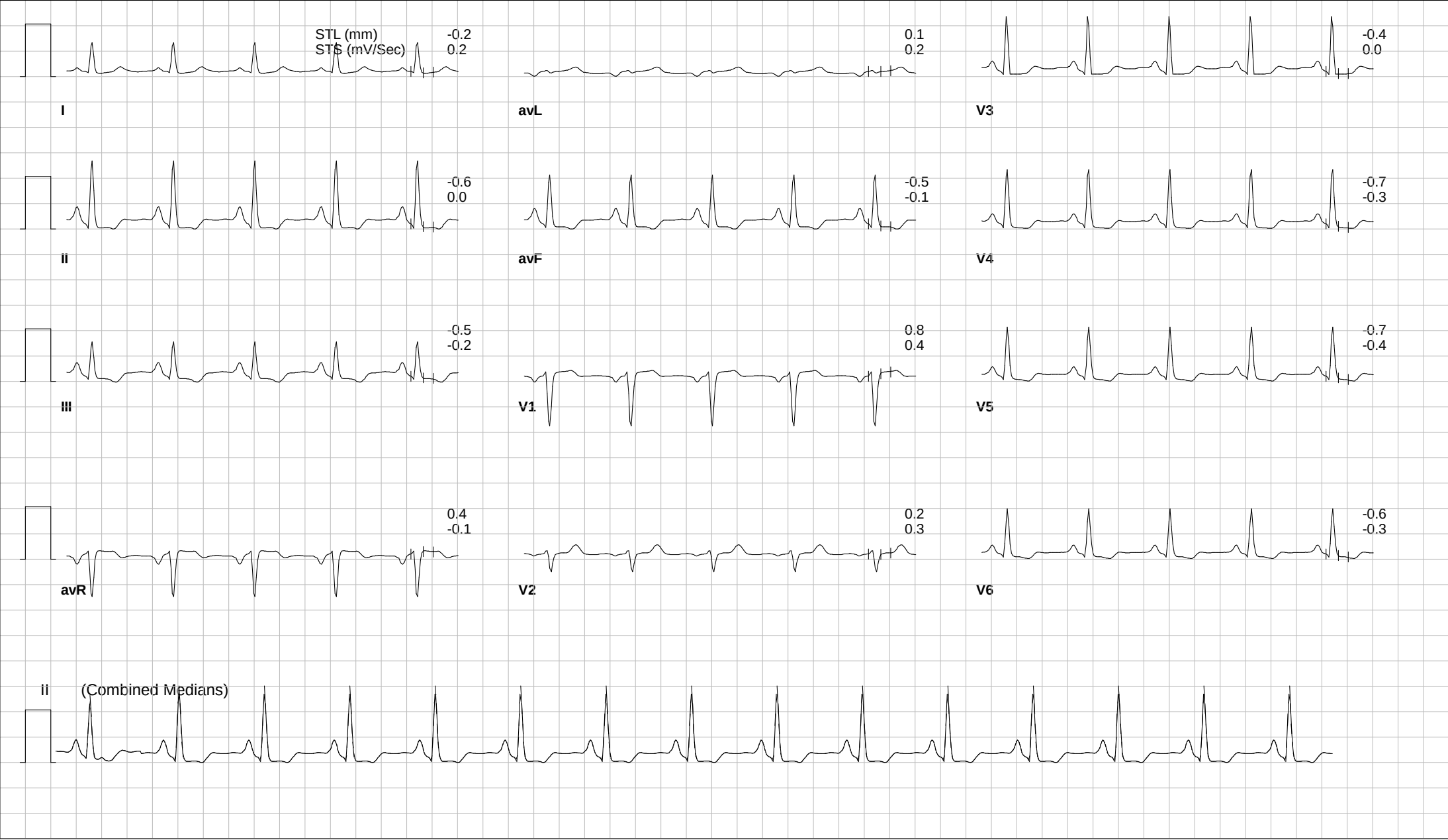


Date: 01 - 09 - 2025 METs : 4.0 HR : 97 Target HR : 57% of 169 BP : 160/90 Post J @80mSec ExTime: 02:25 Speed: 0.0 mph Grade : 00.00 % 25 mm/Sec. 1.0 Cm/mV





Date: 01 - 09 - 2025 METs : 4.0 HR : 92 Target HR : 54% of 169 BP : 150/90 Post J @80mSec ExTime: 02:25 Speed: 0.0 mph Grade : 00.00 % 25 mm/Sec. 1.0 Cm/mV





E-Mail:

255631 / ASMITA DASGUPTA / 51 Yrs / F / 158 Cms / 56 Kg

Date: 01 - 09 - 2025

Refd By : SELF Examined By:

NonCardiacPain Angina /Non-Hypercholestromia/Non-Diabetic/Negative Estrogen/Non-Athlete

Stage	Time	Duration	Speed(mph)	Elevation	METs	Rate	% THR	BP	RPP	PVC	Comments
Supine	00:09	0:09	00.0	00.0	01.0	079	47 %	150/90	118	00	
Standing	00:15	0:06	00.0	00.0	01.0	079	47 %	150/90	118	00	
HV	00:19	0:04	00.0	00.0	01.0	079	47 %	150/90	118	00	
Warm Up	00:23	0:04	01.0	00.0	01.0	083	49 %	150/90	124	00	
ExStart	00:52	0:29	01.0	00.0	01.0	102	60 %	150/90	153	00	
PeakEx	03:17	2:25	01.7	10.0	04.0	165	98 %	170/90	280	00	
Recovery	03:47	0:30	01.1	00.0	04.0	165	98 %	170/90	280	00	
Recovery	04:17	1:00	01.1	00.0	04.0	164	97 %	170/90	278	00	
Recovery	05:17	2:00	00.0	00.0	04.0	133	79 %	170/90	226	00	
Recovery	06:17	3:00	00.0	00.0	04.0	104	62 %	170/90	176	00	
Recovery	07:17	4:00	00.0	00.0	04.0	092	54 %	160/90	147	00	
Recovery	08:17	5:00	00.0	00.0	04.0	085	50 %	160/90	136	00	
Recovery	09:17	6:00	00.0	00.0	04.0	105	62 %	160/90	168	00	
Recovery	10:17	7:00	00.0	00.0	04.0	097	57 %	160/90	155	00	
Recovery	10:35	7:18	00.0	00.0	04.0	092	54 %	150/90	138	00	

FINDINGS :

Exercise Time : 02:25

Initial HR (ExStrt) : 102 bpm 60% of Target 169

Max HR Attained 165 bpm 98% of Target 169

Initial BP (ExStrt) : 150/90 (mm/Hg)

Max BP Attained 170/90 (mm/Hg)

Max WorkLoad Attained : 4 Poor response to induced stress

Max ST Dep Lead & Avg ST Value : II & -2.0 mm in ExStart

Duke Treadmill Score : 02.4

Test End Reasons : Test Complete, Heart Rate Achieved

REPORT :

Doctor : DR.ANUP SHYAMAL MBBS,MD

Patient Name : Mrs. ASMITA DASGUPTA**Age/ Gender** : 51 Yrs / Female**UHID** : FSB.R.0000018456**OP Visit No.** : FSBROPV29438**Printed On** : Sep 01, 2025, 1:45 PM**Advised/Pres Doctor** : --**Department** : Cardiology**Qualification** : --**Referred By** : Self**Registration No.** : --**Employer Id** : 123

DEPARTMENT OF CARDIOLOGY

ELECTROCARDIOGRAM /ECG**Rate** : 60 bpm**Rhythm** : Normal Sinus Rhythm**P-wave** : Normal**Axis** : Normal**PR-interval** : 136 ms**QRS Interval** : 75 ms**QT/QTc** : 403/404 ms**ST-segment** : No specific ST segment changes**QRS complex**: Normal**T-wave** : No specific 'T' wave changes**Q-wave** : Insignificant**Ectopic** : Nil**IMPRESSION :**

- > Normal Sinus Rhythm.
- > No specific ST-T changes.

Note : Please clinically correlate.

---End Of The Report---



Dr. ANUP SHYAMAL
MBBS,MD,CONSULTANT CARDIOLOGIST
57185(WBMC)

Patient Name : Mrs. ASMITA DASGUPTA**Age/ Gender** : 51 Yrs / Female**UHID** : FSB.R.0000018456**OP Visit No.** : FSBROPV29438**Printed On** : Sep 01, 2025, 2:49 PM**Advised/Pres Doctor** : --**Department** : Cardiology**Qualification** : --**Referred By** : Self**Registration No.** : --**Employer Id** : 123

DEPARTMENT OF CARDIOLOGY

TREADMILL TEST / TMT

Resting ECG showing normal sinus rhythm.

During TMT patient showed normal chronotropic & hemodynamic response.

Test End Reason :Target heart rate achieved.

Exercise tolerance was poor.(METs = 4)

Patient was asymptomatic during exercise.

No Significant ST-T changes noted during test.

No Pathological arrhythmia was detected during the procedure.

IMPRESSION:

TMT is negative for provokable myocardial ischemia.

Note : Please clinically correlate.

---End Of The Report---



Dr. ANUP SHYAMAL
MBBS,MD,CONSULTANT CARDIOLOGIST
57185(WBMC)

Patient Name : Mrs. ASMITA DASGUPTA**Advised/Pres Doctor** : --**Age/ Gender** : 51 Yrs / Female**Department** : Radiology**UHID** : FSB.R.0000018456**Qualification** : --**OP Visit No.** : FSBROPV29438**Referred By** : Self**Printed On** : Sep 01, 2025, 11:55 AM**Registration No.** : --**Employer Id** : 123

DEPARTMENT OF RADIOLOGY

Ultrasound Whole Abdomen (Female)

LIVER : Liver is *normal* *in* size with normal parenchyma echopattern . No focal intra-hepatic lesion detected.

Intra-hepatic biliary radicals are not dilated .Portal vein is normal in calibre measures 0.9 cm .

Common bile duct is not dilated 0.6 cm .

GALL BLADDER : Shows no mass or calculus.

PANCREAS : Pancreas is normal in size and echopattern.

SPLEEN : Spleen is normal in size (9.2 cm) & normal echopattern.

KIDNEYS : Both kidneys are normal in position, size and outline.

Cortico-medullary differentiation of both kidneys is maintained.

Central sinus echoes are compact.

Right kidney measures 9.3 cm . Left kidney measures 10.2 cm .

Visualized parts of retroperitoneum do not reveal any lymphadenopathy.

URINARY BLADDER : Urinary bladder is normal in wall thickness with clear contents.

Patient Name : Mrs. ASMITA DASGUPTA**Age/ Gender** : 51 Yrs / Female**UHID** : FSB.R.0000018456**OP Visit No.** : FSBROPV29438**Printed On** : Sep 01, 2025, 11:55 AM**Advised/Pres Doctor** : --**Department** : Radiology**Qualification** : --**Referred By** : Self**Registration No.** : --**Employer Id** : 123

DEPARTMENT OF RADIOLOGY

UTERUS : Anteverted uterus is normal in size , shape, and echotexture with Shows normal myometrial echogenecity . Uterus measures 6.3 x 3.7 x 4.0 cm . Endometrium is central and of normal thickness measures 0.8 cm . Cervical canal is normal .

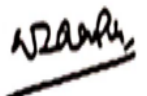
OVARIES : Both ovaris are normal in size & shape . Parenchyma of both ovaries are normal . Right ovary measures 1.9 x 1.7 cm . Left ovary measures 2.5 x 2.1 cm .

No free fluid detected in pouch of Douglas and Morissons pouch.

IMPRESSION : Normal scan .

ADV : Clinically correlation and follow up.

---End Of The Report---



Dr. M NURUZZAMAN
MBBS, DMRD, MD
45591(WBMC)



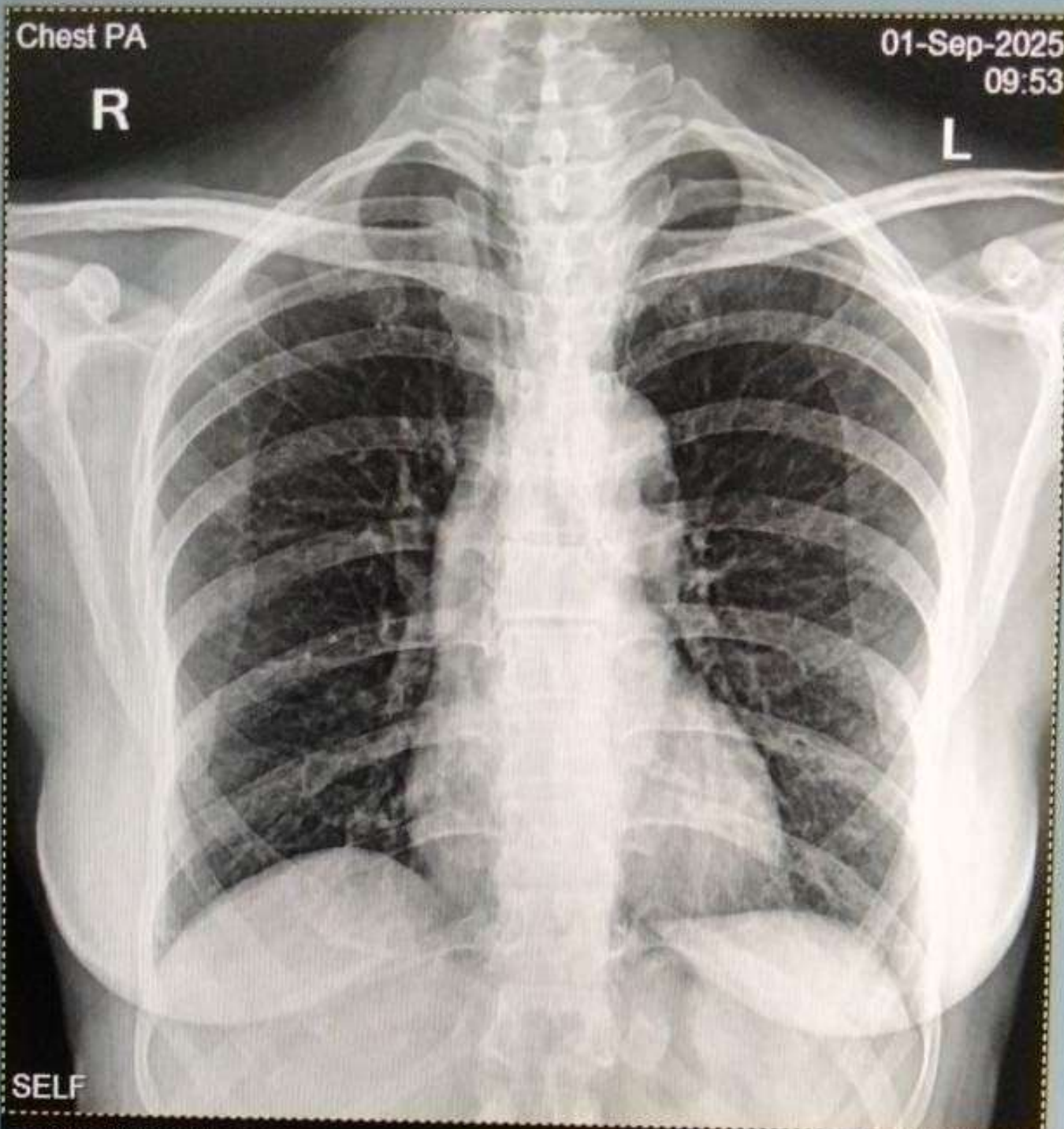
Chest PA

01-Sep-2025

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SELF

00, ASMITA DASGUPTA, F, 51 years

APOLLO CLINIC (Licensee-SGD 369) ,44/2,SARAT BOSE ROAD

Patient Name : Mrs. ASMITA DASGUPTA**Advised/Pres Doctor** : --**Age/ Gender** : 51 Yrs / Female**Department** : Radiology**UHID** : FSB.R.0000018456**Qualification** : --**OP Visit No.** : FSBROPV29438**Referred By** : Self**Printed On** : Sep 01, 2025, 12:20 PM**Registration No.** : --**Employer Id** : 123

DEPARTMENT OF RADIOLOGY

X-RAY CHEST PA

Lung parenchyma shows no focal lesion.

Bronchovascular lung markings are within normal limits.

Both of hila are normal in size, density and position.

Domes of diaphragm are smoothly outlined.

Bilateral costophrenic angles are clear.

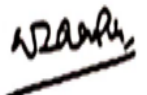
Cardiac size appears within normal limits.

Bony thorax reveals no definite abnormality.

IMPRESSION: Normal Study.

Note: Please clinically correlate.

---End Of The Report---



Dr. M NURUZZAMAN

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